

Amendment No. 1 to HB2539

Terry
Signature of Sponsor

AMEND Senate Bill No. 2227*

House Bill No. 2539

by deleting all language after the enacting clause and substituting:

SECTION 1.

(a) Tennessee Code Annotated, Title 4, Chapter 29, Part 2, is amended by deleting the following:

Board of dispensing opticians, created by § 63-14-101;

Committee for clinical perfusionists, created by § 63-28-112;

Professional art therapist advisory committee of the board of examiners in psychology, created by § 63-11-401;

(b) Tennessee Code Annotated, Title 4, Chapter 29, Part 2, is amended by deleting "Board of examiners in psychology, created by § 63-11-101;" and substituting "Board of examiners in psychology, created by § 63-11-102;".

SECTION 2. Tennessee Code Annotated, Section 4-29-248(a), is amended by deleting subdivisions (34) and (35).

SECTION 3. Tennessee Code Annotated, Section 4-29-249(a), is amended by deleting subdivisions (5), (9), (12), (33), and (39).

SECTION 4. Tennessee Code Annotated, Section 8-8-201(b)(1), is amended by deleting "63-17-219" and substituting "63-17-217".

SECTION 5. Tennessee Code Annotated, Section 33-3-114, is amended by deleting "63-11-213" and substituting "63-11-211".

SECTION 6. Tennessee Code Annotated, Section 37-1-411, is amended by deleting "§ 63-11-213" and substituting "§ 63-11-211".

SECTION 7. Tennessee Code Annotated, Section 37-3-703(d), is amended by deleting the subsection and substituting:

(d) The state of Tennessee shall conduct ongoing evaluations of the healthy start pilot project and publish data on or before December 31 of each year. Each state agency that provides services to children shall make available nonidentifying information about healthy start participants for the purpose of conducting the evaluation. The report must include the following information for the preceding fiscal year:

- (1) The number of families receiving services through the project;
- (2) The average cost of services provided under the project;
- (3) The performance measures achieved by program participants; and
- (4) An overall statement of the achievements and progress of the project during the preceding fiscal year, along with recommendations for improvement or expansion.

SECTION 8. Tennessee Code Annotated, Section 52-3-114, is amended by deleting "63-11-213" and substituting "63-11-211".

SECTION 9. Tennessee Code Annotated, Section 53-10-309, is amended by deleting the first two sentences.

SECTION 10. Tennessee Code Annotated, Section 62-4-109(a)(1), is amended by deleting "§ 63-18-102" and substituting "§ 63-33-201".

SECTION 11. Tennessee Code Annotated, Section 63-1-104(b), is amended by deleting "and shall be signed by the executive officer and by the members of the state board charged with the duty by law of issuing the preliminary certificates".

SECTION 12. Tennessee Code Annotated, Section 63-1-120, is amended by deleting the section and substituting the following:

(a)

- (1) With respect to a person required to be licensed, permitted, certified, or authorized by a board, council, committee, agency, or regulatory program

created pursuant to chapters 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 16, 17, 19, 22, 23, 25, 29, and 33 of this title and to title 68, chapter 140, attached to the division of health related boards, such board, council, committee, agency, or regulatory program may:

(A) Deny an application for licensure, certification, permit, or authorization;

(B) Permanently withhold issuance of licensure, certification, permit, or authorization;

(C) Suspend, limit, or restrict previously issued licensure, certification, permit, or authorization; or

(D) Otherwise discipline a holder of a license, certificate, permit, or authorization, if the applicant, licensee, or certificate or permit holder has been disciplined by another state of the United States for any acts or omissions that would constitute grounds for discipline of a person licensed, certified, permitted, or authorized in this state.

(2) A certified copy of the initial order, final order, or other equivalent document memorializing the disciplinary action from the disciplining state constitutes prima facie evidence of a violation of this section and is sufficient grounds upon which to deny, restrict, or condition the license, permit, or certificate renewal or application, or to otherwise discipline a licensee licensed in this state.

(b) In disciplinary actions against individuals holding a license, certificate, permit, or authorization in this state at the time of a disciplinary action in another reporting state, in the absence of justifying evidence to the contrary, there is a rebuttable presumption that the sanction proposed in the proceeding is comparable to that in the reporting state; however, such presumption does not exist for those who are applying for licensure, certification, permit, or authorization in this state during or after the time the disciplinary

action in the other state is pending or has become final. If a board, council, committee, agency, or regulatory program created pursuant to chapters 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 16, 17, 19, 22, 23, 25, 29, and 33 of this title or title 68, chapter 140 denies, restricts, or conditions a licensure, certification, permit, or authorization based on a disciplinary action in another state, then the applicant, upon written request, filed within thirty (30) days of the date of the action on the application, is entitled to a contested case hearing.

SECTION 13. Tennessee Code Annotated, Section 63-1-125(b)(1), is amended by deleting the subdivision and substituting:

(1) "Board" means a health-related board created in this title or title 68 and includes:

(A) The board for professional counselors, marital and family therapists, and clinical pastoral therapists, created by § 63-22-101;

(B) The board of alcohol and drug abuse counselors, created by § 68-24-601;

(C) The board of occupational therapy, created by § 63-13-216;

(D) The board of examiners in psychology, created by § 63-11-102; and

(E) The board of social work licensure, created by § 63-23-101; and

SECTION 14.

(a) Tennessee Code Annotated, Section 63-1-126(a)(4), is amended by deleting the subdivision and substituting:

(4) "Healthcare practitioner" or "practitioner" means any person required to be licensed, permitted, certified, or authorized:

(A) Under this title by a board or committee under the division of health-related boards specified in § 68-1-101(a)(8), who has humans for patients;

(B) Under this title by the commissioner of health;

(C) Under title 68, chapter 24, part 6; or

(D) Under title 68, chapter 140.

(b) Tennessee Code Annotated, Section 63-1-126(a)(4)(B), is amended by deleting the subdivision.

SECTION 15.

(a) Tennessee Code Annotated, Section 63-1-144(a), is amended by deleting "council" wherever it appears and substituting "council, agency,".

(b) Tennessee Code Annotated, Section 63-1-144(b), is amended by deleting the subsection and substituting:

(b) The agency, board, committee, council, or division shall include in any order in which the payment of costs has been assessed an amount that is the maximum amount owed by the license or certificate holder at the time the order is entered. Prior to the expiration of sixty (60) days from the effective date of the order, the agency, board, committee, council, or division shall send to the license or certificate holder, by certified mail, return receipt requested, and by regular United States mail, a final costs assessment that does not exceed the maximum amount in the order.

(c) Tennessee Code Annotated, Section 63-1-144(c)(1), is amended by deleting "board, committee, or council" and substituting "agency, board, committee, council, or division".

SECTION 16. Tennessee Code Annotated, Section 63-1-150(a), is amended by deleting "63-11-220" and substituting "63-11-213".

SECTION 17. Tennessee Code Annotated, Section 63-1-153, is amended by deleting the section.

SECTION 18. Tennessee Code Annotated, Section 63-1-164(f), is amended by deleting the subsection.

SECTION 19. Tennessee Code Annotated, Section 63-1-177, is amended by deleting "24,".

SECTION 20. Tennessee Code Annotated, Section 63-1-314, is amended by deleting the section.

SECTION 21. Tennessee Code Annotated, Section 63-5-107(c)(1)(A), is amended by deleting the subdivision and substituting:

(A)

(i) Each person licensed as a dentist or dental hygienist, or registered as a dental assistant, is required to attend and complete during each two-year licensure cycle, except for the initial licensure cycle and the first licensure renewal cycle, the number of hours of board-approved continuing education courses that are prescribed by the board, subject to a minimum of twenty-four (24), and a maximum of sixty (60), clock hours. However, for the purposes of the biennial continuing education requirement for dental hygienists, no more than fifty percent (50%) of such continuing education requirement may be earned from online or web-based courses. Sponsors of such courses shall submit a sufficient description of course content to the board in order to obtain approval.

(ii) The board may approve all, part, or none of the course at its discretion. Additional procedures for implementing this requirement may be adopted by the board in its rules. The board may waive all or part of the continuing education requirements in unusual situations in which an applicant for renewal has demonstrated inability to attend such courses not attributable to fault of the applicant. The board has discretion to require completion of continuing education courses of a type and number of hours satisfactory to the board in cases involving license renewal of dentists or dental hygienists who have not actively practiced or have been retired from practice for more than two (2) years.

(iii) In addition to subdivisions (c)(1)(A)(i) and (ii), all applicants shall provide proof of current CPR certification by an organization approved by the board and maintain such certification unless such requirement is waived by the

board in its discretion; provided, that a dentist who obtains a waiver from the board for CPR requirements shall not practice dentistry unless a dentist, dental hygienist, or dental assistant who is currently certified in CPR is present.

SECTION 22. Tennessee Code Annotated, Section 63-6-105, is amended by deleting the section.

SECTION 23.

(a) Tennessee Code Annotated, Section 63-6-221(h), is amended by deleting "The board shall enter into a memorandum of understanding, contract or other written arrangement with the health facilities commission such that the commission:" and substituting:

The department's investigators are authorized on behalf of the board to do the following:

(b) Tennessee Code Annotated, Section 63-6-221(h)(1), is amended by deleting "Provides" and substituting "Provide".

(c) Tennessee Code Annotated, Section 63-6-221(h)(2), is amended by deleting "Is authorized to require" and substituting "Require".

(d) Tennessee Code Annotated, Section 63-6-221(h)(3), is amended by deleting "Is authorized to initiate" and substituting "Initiate".

(e) Tennessee Code Annotated, Section 63-6-221(h)(4), is amended by deleting "Is authorized to respond" and substituting "Respond".

SECTION 24. Tennessee Code Annotated, Section 63-6-221(i), is amended by deleting the subsection and substituting:

(i) The results of all site surveys must be transmitted to the board. The results must include any requirement for plans of correction, the determination of the acceptability of the submitted plans of correction, and the verification that the plans of correction have been implemented. The board shall make a final determination on certifying the surgical suite for performance of office-based surgeries. The results of site

surveys and board determinations must be shared on a routine basis with the board for licensing health care facilities.

SECTION 25. Tennessee Code Annotated, Title 63, Chapter 6, Part 10, is amended by deleting the part.

SECTION 26. Tennessee Code Annotated, Title 63, Chapter 11, Part 1, is amended deleting the part and substituting:

63-11-101. Chapter definitions.

As used in this chapter:

- (1) "Board" means the board of examiners in psychology, created by § 63-11-102;
- (2) "Commissioner" means the commissioner of health;
- (3) "Department" means the department of health; and
- (4) "Division" means the division of health related boards in the department of health, created by § 63-1-101.

63-11-102. Creation – Membership – Immunity.

(a) There is created a board of examiners in psychology to consist of ten (10) members, nine (9) of whom are appointed by the governor under conditions set forth in subsection (b).

(b)

- (1) Two (2) members must be chosen from, and be members of the faculty with the rank of assistant professor or above of, the accredited colleges and universities in the state and be engaged in teaching, research, or administration of psychology.
- (2) Four (4) members must be licensed psychologists.
- (3) One (1) member must be a private citizen who is neither a licensed psychologist nor a licensed psychological examiner or licensed senior

psychological examiner and who has no professional or commercial interest in the practice of psychology.

(4) Two (2) members must be either a licensed psychological examiner or a licensed senior psychological examiner.

(c) The chair of the applied behavior analyst licensing committee of the board of examiners in psychology shall serve as an ex officio voting member of the board.

(d) The board shall perform such duties and exercise such powers as this chapter prescribes and confers upon it.

(e) Members of the board are not liable to civil actions for any act performed in good faith in the performance of their duties as set forth in this chapter.

63-11-103. Appointments – Vacancies.

(a) All appointments are for terms of five (5) years.

(b) Vacancies must be filled for any unexpired terms, and members shall serve until their successors are appointed and have been qualified. Only practicing psychologists must be appointed to succeed practicing psychologists, and only academic psychologists must be appointed to succeed academic psychologists. Board members are ineligible for reappointments for a period of five (5) years following completion of their terms.

(c)

(1) Appointments to the board for the psychologist members may be made from lists of qualified candidates to be furnished to the governor by interested psychology groups, including, but not limited to, the Tennessee Psychological Association. Each list must contain the names of academic psychologists and practicing psychologists. All vacancies occurring in the board may be filled by the governor for the unexpired term from the lists of qualified candidates within thirty (30) days after the vacancy occurs. The governor shall

consult with such interested groups to determine qualified persons to fill positions on the board.

(2) In making appointments to the board, the governor shall strive to ensure that at least one (1) person serving on the board is sixty (60) years of age or older.

63-11-104. Oath of office – Records.

(a) Immediately and before entering upon the duties of the office, the members of the board shall take the constitutional oath of office and file the same in the office of the governor.

(b) The board shall have available for the governor adequate records of its proceedings and of all licenses and certificates.

63-11-105. Organization – Administration.

(a) The board shall elect annually a chair and vice chair.

(b) Each member must receive all necessary expenses incident to conducting the business of the board and, in addition thereto, be entitled to a per diem of one hundred dollars (\$100) for each day's service in conducting the business of the board. All reimbursements for travel expenses must be in accordance with the comprehensive travel regulations as promulgated by the department of finance and administration and approved by the attorney general and reporter.

(c) The board shall hold at least one (1) regular meeting each year. Called meetings may be held at the discretion of the chair or at the written request of two (2) members of the board.

(d) The board shall adopt a seal, which must be affixed to all certificates issued by the board.

(e) Five (5) members of the board at all times constitute a quorum. An action taken by the board requires the majority of the quorum present.

(f) The board is empowered to hire such assistance as is necessary to carry on its activities, within the limits of funds available to the board, and is empowered to accept grants from foundations or institutions to develop tests and carry out its functions.

(g) The board shall receive administrative support from the division.

(h) The board shall establish continuing educational requirements for all licenses created in this chapter.

63-11-106. Powers and duties.

(a) The board has the following powers and duties in addition to the powers and duties granted to or imposed upon it by this chapter:

(1) To prescribe rules for examination of candidates for a license created by this chapter;

(2) To ascertain the qualifications and fitness of applicants for all licenses created by this chapter;

(3) To issue licenses to applicants who successfully meet the qualifications for licensure under this chapter and the board's rules;

(4) To receive complaints and review issues related to complaints and disciplinary actions involving a person licensed under this chapter;

(5) To conduct investigations, as appropriate, into allegations against licensees of the board;

(6) To issue private letter rulings in accordance with § 63-1-174;

(7) To make rules and establish fees as are necessary to carry out and make effective this chapter; and

(8) To administer oaths, summon witnesses, and take testimony in all matters relating to the board's duties.

(b) All rules promulgated by the board must comply with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(c) The board shall use the division-employed investigators, inspectors, and agents, or use any other means necessary, to bring about and maintain the rigid administration and enforcement of this chapter and all laws regulating the practice of a profession created by this chapter.

63-11-107. License – Application.

(a) Applicants for a license created by this chapter shall submit an application on the form created by the board and any required fees to the division.

(b) In addition to the specific requirements for licensure in this chapter, the application must include evidence that the applicant meets the following requirements:

(1) The applicant is of good moral character and conducts the applicant's professional activities in accordance with the accepted professional and ethical standards;

(2) The applicant has not had a professional credential refused, revoked, suspended, or restricted and does not have a complaint, allegation, or investigation pending in any regulatory jurisdiction in the United States or in another country for unprofessional conduct, and has not committed an act that would constitute grounds for discipline of a license under § 63-11-109; and

(3) The applicant is either a United States citizen or a qualified alien, as defined in § 4-58-102.

(c) The division and applicant shall comply with the criminal background requirements of § 63-1-116.

(d) Each applicant seeking licensure must pay all fees as set by the board, including, but not limited to, nonrefundable application fees and examination fees.

(e) All licensees must comply with § 63-1-109.

(f) Portions of an application involving an applicant's health condition, including mental health conditions and substance use disorders, and records involving an

applicant's health condition, are confidential, privileged, and not public records subject to inspection by citizens of this state, as described in § 10-7-503.

(g)

(1) When the board receives a completed application for initial licensure, or a completed application for licensure, from an applicant who is licensed in another state or territory of the United States or in the District of Columbia, then the board shall, within sixty (60) days from the date the board receives the completed application:

(A) Render a decision on the application; or

(B) Inform the applicant of the need to appear before the board.

(2) As used in this subsection (g), "completed application" means an application that satisfies all statutory and board rule requirements.

63-11-108. License renewal – Retirement.

(a) A license issued under this chapter is valid for twenty-four (24) months from the date of issuance and is renewable biennially at the end of the birth month of the licensee. The division, with the approval of the commissioner, shall establish a system of license or certification renewals at alternative intervals that allow for the distribution of the license or certification workload as uniformly as is practicable throughout the calendar year. Licenses or certificates issued under the alternative method are valid for twenty-four (24) months and expire on the last day of the last month of the license or certification period.

(b) In addition to any requirements of this chapter creating the license, a licensee requesting renewal of a certificate must provide the following to the division in advance of the expiration date of the license:

(1) A completed application on a form provided by the division and payment of the fees set by the board; and

(2) A certified or sworn statement that the licensee:

(A) Has completed all the continuing education hours required for the licensee, whether required by statute or rule, prior to the expiration date of the license; and

(B) Has not had a professional credential revoked, suspended, or restricted by a certification board, and does not have a complaint, allegation, or investigation pending in this state or in any regulatory jurisdiction for unethical or unprofessional conduct.

(c) If a licensee fails to pay the license renewal fee within sixty (60) days after it becomes due, as provided in this section, the licensee must apply for a new license rather than renew the existing license.

(d) A licensee who has lost the ability to renew pursuant to subsection (c) may make application in writing to the board for the reinstatement of the license, and, upon good cause shown, the board, in its discretion, may reinstate the license upon payment of all past due renewal fees and upon further payment of a sum set by the board.

(e) Applicants and licensees shall notify the board of the following within thirty (30) days of their occurrence:

(1) A change of name, address, or other vital information;

(2) The filing of a criminal charge or civil action against the applicant or licensee;

(3) The initiation of disciplinary charges, investigations, findings, or sanctions by a healthcare organization, federal or state agency, or other professional association against the applicant or licensee; and

(4) Any other change in information provided by the applicant or licensee to the division.

(f) A person licensed or certified to practice by this chapter who has retired or may retire on or after the effective date of this act from such practice in this state must not be made to register as required by this chapter if the person files with this board an

affidavit on a form to be furnished by the board, which affidavit must state the date on which the person retired from such practice and such other facts as tend to verify the retirement as the board deems necessary. If the person thereafter reengages in such practice in this state, then the person must apply for registration with the board as provided by this chapter and meet other requirements as may be set by the board.

(g) For the purpose of effecting service of process upon a licensee, the board may notify the licensee by certified mail, return receipt requested, at the address on file with the division.

63-11-109. License denial, suspension, or revocation – Disciplinary grounds – Hearing.

(a) The board has the power:

(1) To deny an application for a license to an applicant who applies for the same through reciprocity or otherwise;

(2) To permanently or temporarily withhold issuance of a license;

(3) To issue a license contingent upon such circumstances or requirements as the board may determine;

(4) To issue a conditional license that is limited or restricted for such time and in such manner as the board may determine, including a limited license of temporary duration for applicants out of practice for an extended period of time;

(5) To issue discipline to an applicant or licensee, or on a previously issued license, including, but not limited to, private censures and warnings, public reprimands, limits, restrictions, conditions, or suspension periods, for such time and in such manner as the board may deem proper in its discretion;

(6) To discipline a licensee who has been disciplined by another state of the United States for any acts or omissions that would constitute grounds for discipline if licensed in this state, following the procedure in § 63-1-120; or

(7) To revoke, or permanently revoke, a license.

(b) The grounds upon which the board may exercise the powers enumerated in subsection (a) include, but are not limited to:

- (1) Engaging in unprofessional, dishonorable, or unethical conduct;
- (2) Making false statements or representations, being guilty of fraud or deceit in obtaining admission to practice, or being guilty of fraud or deceit in the course of professional practice;
- (3) Violating or attempting to violate, directly or indirectly, or assisting in or abetting the violation of, or conspiring to violate, a criminal statute of this state, this chapter, a rule promulgated by the board, or a lawful order of the board issued pursuant to rules;
- (4) Engaging in gross professional liability, or a pattern of continued or repeated professional liability, ignorance, negligence, or incompetence, in the course of practice;
- (5) Being habitually intoxicated or personally misusing drugs, or using intoxicating liquors, narcotics, controlled substances, controlled substance analogues, or other drugs or stimulants, in such manner as to adversely affect the person's ability to safely practice in the profession;
- (6) Willfully betraying a professional secret;
- (7) Advertising the professional practice with untrue or misleading statements or causing the publication or circulation of fraudulent advertising relative to any disease, human ailment, or condition;
- (8) Willfully violating the rules that may be promulgated by the board to regulate advertising by practitioners who are under the jurisdiction of the board;
- (9) Being convicted of a felony, an offense under state or federal laws relative to drugs or the professional practice, an offense involving moral turpitude, or an offense for which the licensee or applicant is required to register as a sexual offender or violent sexual offender pursuant to the Tennessee Sexual

Offender and Violent Sexual Offender Registration, Verification and Tracking Act of 2004, compiled in title 40, chapter 39, part 2;

(10) Engaging in professional practice when mentally or physically unable to safely do so;

(11) Making or signing in one's professional capacity a certificate that is known to be false at the time one makes or signs the certificate;

(12) Offering, undertaking, or agreeing to cure or treat a disease, injury, ailment, or infirmity by a secret means, method, device, or instrumentality;

(13) Giving, receiving, or aiding or abetting in the giving or receiving of rebates, either directly or indirectly;

(14) Engaging in practice under a false or assumed name or the impersonation of another practitioner of a like, similar, or different name;

(15) Practicing a level of psychology inappropriate or beyond the scope of the particular license or licenses or certification held by the licensee or certified person; and

(16) Engaging in any other action deemed to be grounds for disciplinary action under rules promulgated by the board.

(c) All proceedings for disciplinary action against a licensee under this chapter must be conducted in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(d) An action taken under this section is subject to the applicable provisions of the Fresh Start Act, compiled in chapter 1, part 1 of this title.

SECTION 27. Tennessee Code Annotated, Title 63, Chapter 11, Part 2, is amended by deleting the part and substituting:

63-11-201. Levels of psychological practice.

(a) Five (5) levels of psychological practice are defined for the purposes of this chapter. Such levels are to be known and referred to as:

- (1) Psychological examiner;
- (2) Senior psychological examiner;
- (3) Psychologist;
- (4) Certified psychological assistant; and
- (5) Certified psychological testing technician.

(b) An individual who engages in the practice of psychology as a psychological examiner, senior psychological examiner, or psychologist must possess a valid license under this chapter.

(c) A person engaged in the practice of psychology as a certified psychological assistant or certified psychological testing technician must possess a valid certificate to practice under this chapter.

(d)

(1) This section does not prohibit a charitable clinic from employing or contracting with a psychologist if the contractual relationship between the psychologist and the charitable clinic is evidenced by a written contract, job description, or documentation, containing language that does not restrict the psychologist from exercising independent professional judgment in diagnosing and treating patients.

(2) As used in this subsection (d), "charitable clinic" means an entity that meets the following standards:

(A) Has received a determination of exemption from the internal revenue service under 26 U.S.C. § 501(c)(3) or is a distinct part of an entity that has received such a determination of exemption;

(B) Has clinical facilities located in this state;

(C) Has a primary mission to provide health care or dental care services to low-income, uninsured, or underserved individuals;

(D) Provides one (1) or more of the following services for free or at a discounted rate:

- (i) Medical care;
- (ii) Dental care;
- (iii) Mental health care; or
- (iv) Prescription medications;

(E) Utilizes volunteer healthcare professionals and nonclinical volunteers; and

(F) Is not required to be licensed under § 68-11-202(a)(1).

63-11-202. Practice of a psychological examiner.

(a) A person practices as a psychological examiner within the meaning of this chapter when the person claims to be a psychological examiner or renders to individuals or to the public health-related clinical activities or services involving the application of recognized principles, methods, and procedures of the science and profession of psychology, such as interviewing or administering and interpreting tests of mental abilities, aptitudes, interests, and personality characteristics, for such purposes as psychological evaluation or for educational or vocational selection, guidance, or placement. The psychological examiner shall practice the following health-related clinical activities or services only under qualified supervision provided by a psychologist or a senior psychological examiner:

- (1) Overall personality appraisal or classification;
- (2) Personality counseling;
- (3) Psychotherapy;
- (4) Behavior analysis; or
- (5) Personality readjustment techniques.

(b) This section does not expand or restrict the scope of practice for a psychological examiner.

(c) Those psychological examiners rendering health-related clinical activities or services who have been duly licensed prior to July 1, 1991, who request to the board in writing or via online application, are senior psychological examiners. Those duly licensed psychological examiners rendering health-related clinical activities or services licensed after July 1, 1991, qualify for senior psychological examiner licensure upon completion of at least five (5) years of applied experience from the date their temporary permit was first issued by the board and upon completion of two hundred (200) hours of post-licensure continuing education units distributed by categories, according to the November 1997 rules governing continuing education units. A person practices as a senior psychological examiner within the meaning of this chapter when the person claims to be a senior psychological examiner or renders to individuals or to the public for remuneration a service involving the application of recognized principles, methods, and procedures of the science and profession of psychology. A senior psychological examiner, while possessing a different scope of practice than a psychologist, is considered a health service provider and able to engage in the following designated areas of health-related psychological practice without supervision:

(1) Interviewing or administering and interpreting tests of mental abilities, aptitudes, interests, and personality characteristics for such purposes as psychological evaluation or for educational or vocational selection, guidance, or placement;

(2) Overall personality appraisal or classification, psychological testing, projective testing, and evaluations for disability or vocational purposes;

(3) Diagnosis of nervous and mental disorders;

(4) Personality counseling;

(5) Psychotherapy;

(6) Behavior analysis;

(7) Personality readjustment techniques; or

(8) Supervision of a psychological examiner.

63-11-203. Practice of a psychologist.

(a) As used in this part, "practice of psychology":

(1) Means, without regard to whether payment is received for services rendered, the observation, description, evaluation, interpretation, and modification of human behavior by the application of psychological principles, methods, and procedures for the purpose of assessing, preventing, or eliminating symptomatic, maladaptive, or undesired behavior and of enhancing interpersonal relationships, work and life adjustment, personal effectiveness, behavioral health, and mental health of individuals, families, groups, and the public; and

(2) Includes:

(A) Psychological testing and the evaluation or assessment of personal characteristics, such as intelligence, personality, abilities, interests, aptitudes, and neuropsychological functioning;

(B) Counseling, psychoanalysis, psychotherapy, hypnosis, biofeedback, and behavior analysis and therapy;

(C) Psychological diagnosis and treatment of mental, emotional, and nervous disorders or disabilities; alcoholism and substance abuse; disorders of habit or conduct; and psychological aspects of physical illness, accident, injury, or disability;

(D) Case management and utilization review of psychological services;

(E) Psychoeducational evaluation, therapy, remediation, and consultation;

(F) Forensic evaluation; provided, that a person licensed under this chapter performing forensic evaluation is considered to be acting

within their lawful scope of practice and the practice of forensic evaluation is not exclusive to individuals licensed under this chapter;

(G) Parent coordination; provided, that a person licensed under this chapter performing parent coordination is considered to be acting within their lawful scope of practice and the practice of parent coordination is not exclusive to individuals licensed under this chapter; and

(H) Telepsychology, which means the practice of psychology via electronic communications technology by a person licensed under this chapter.

(b) A person represents to be a psychologist if that person uses a title or description of services incorporating the words "psychology," "psychological," or "psychologist," if the person possesses expert qualification in any area of psychology or if that person offers to the public or renders to individuals or to groups of individuals services pursuant to the practice of psychology.

63-11-204. Psychotherapy limitations – Conduct outside psychological practice prohibited.

(a) Sections 63-11-201 — 63-11-203 do not permit the use of those forms of psychotherapy that involve the administration or prescription of drugs or electroshock or in any way infringe upon the practice of medicine as defined in the laws of this state.

(b) A psychologist, psychological examiner, senior psychological examiner, certified psychological assistant, or certified psychological testing technician who engages in psychotherapy shall establish and maintain effective intercommunication with a psychologically oriented physician, usually a psychiatrist, to make provision for the diagnosis and treatment of medical problems by a physician with an unlimited license to practice the healing arts in this state.

(c) A psychologist, psychological examiner, senior psychological examiner, certified psychological assistant, or certified psychological testing technician shall not attempt to diagnose, prescribe for, treat, or advise a client with reference to problems or complaints falling outside the boundaries of psychological practice.

(d) Sections 63-11-203 and 63-11-208(c) and (d) are subject to the limitations of this section.

63-11-205. Use of titles.

(a)

(1) Individuals and organizations shall not present themselves, or be presented, to the public by a title incorporating the name "psychological," "psychologist," or "psychology" other than those licensed as psychologists, psychological examiners, senior psychological examiners, certified psychological assistants, or certified psychological testing technicians.

(2) A psychological scientist employed by a recognized research laboratory, college, or university may use the academic or research title conferred upon the psychological scientist by the administration of the laboratory, college, or university.

(3) Persons licensed by the state board of education in areas of school psychology, school psychologist, or school psychological services worker and employed as such in the department of education or in a public or nonpublic elementary or secondary school are not required to be licensed under this part while performing in the course of their employment. This chapter does not apply to persons licensed by the state board of education in areas of school psychology, school psychologist, or school psychological services worker who are employed as such in the department of education or in a public or nonpublic elementary or secondary school while performing in the course of their employment.

(b) This section does not permit such persons to offer their services to any other persons or organizations as consultants or to accept remuneration for psychological services other than that of their institutional salaries unless they have been licensed or certified under this chapter.

(c) Visiting lecturers from recognized laboratories, colleges, or universities are exempt from this section and may utilize their academic or research title when presenting lectures to similar institutions or organizations.

(d) Students of psychology, psychological interns, and other persons preparing for the profession of psychological examiner, senior psychological examiner, certified psychological assistant, certified psychological testing technician, or psychologist under qualified supervision in recognized training institutions or facilities may be designated by such titles as "psychological intern," "psychological trainee," or others clearly indicating such training status.

(e) Psychologists with competency in areas other than the provision of health services retain the right to use the title of "licensed psychologist" with competence in areas other than health services and to practice within their area of competency.

63-11-206. Unauthorized practice – Exemptions – Temporary licenses.

(a) A person who engages in the practice of psychology and does not then possess in full force and virtue a valid license to practice as a psychological examiner, psychologist, or senior psychological examiner, or a valid certificate to practice as a psychological assistant or psychological testing technician, under this chapter commits a Class B misdemeanor.

(b) This chapter does not prevent members of other recognized professions who are licensed, certified, or regulated under the laws of this state from rendering services consistent with their professional training and code of ethics as long as they do not represent themselves to be psychologists, psychological examiners, senior psychological examiners, certified psychological assistants, or certified psychological

testing technicians. Duly recognized members of the clergy are not restricted from functioning in their ministerial capacity as long as they do not represent themselves to be psychologists, psychological examiners, senior psychological examiners, certified psychological assistants, or certified psychological testing technicians.

(c)

(1) Students of psychology, psychological interns, and other persons preparing for the profession of psychological examiner, psychologist, senior psychological examiner, certified psychological assistant, or certified psychological testing technician may perform as a part of their training the functions in §§ 63-11-202 and 63-11-203, but only under qualified supervision.

(2) A person practices as a certified psychological assistant within the meaning of this chapter when the person claims to be a certified psychological assistant or renders to individuals or to the public for remuneration a service involving the application of recognized principles, methods, and procedures of the science and profession of psychology and only under the direct employment and supervision of either the psychologist or senior psychological examiner employing the certified psychological assistant or the employing community mental health center or state governmental agency. All work performed by a certified psychological assistant must be supervised by a psychologist or senior psychological examiner as determined by the board.

(3) A person practices as a certified psychological testing technician within the meaning of this chapter when the person claims to be a certified psychological testing technician or renders to individuals or the public for remuneration a service involving the application of recognized principles, methods, and procedures of the science and profession of psychology and only under the supervision of either the psychologist or senior psychological examiner employing the certified psychological testing technician or the employing

community mental health center or state governmental agency. All work performed by a certified psychological testing technician must be supervised by a psychologist or senior psychological examiner as determined by the board.

(d) Use of psychological techniques by governmental institutions and by business and industrial organizations for employment placement, evaluation, promotion, or job adjustment of their own officers or employees or by employment agencies for the evaluation of their own clients prior to recommendation for employment is also specifically allowed; however, an industrial or business firm or corporation shall not sell or offer to the public or to other firms or corporations for remuneration psychological services as specified in §§ 63-11-202 and 63-11-203, unless the services are performed or supervised by individuals duly and appropriately licensed under this chapter.

(e)

(1)

(A)

(i) The board may issue a temporary license to an applicant who has successfully completed the academic course work and training for the license sought, and who has not yet taken the required written examinations.

(ii) A temporary license is valid for a period not to exceed one (1) year. The board may extend the expiration date of a temporary license following a petition from the licensee and finding that extraordinary circumstances exist that necessitate a delay in taking a required written examination prior to the expiration of the initial temporary license period.

(iii) A temporary license is revoked thirty (30) days after the date a temporary licensee receives notice of the licensee's second failure of the examination for professional practice in

psychology (EPPP) or jurisprudence examination. During the thirty-day period prior to revocation of the temporary license, the licensee and the licensee's supervisor shall ensure the notification and transfer of care for the temporary licensee's clients.

(B) The temporary license allows the applicant to perform the functions specified in §§ 63-11-202 and 63-11-203, for which the applicant is seeking licensure under qualified supervision.

(C) In order to receive a temporary license, the applicant must submit a completed application for a temporary license with the applicant's completed application for the desired license and all appropriate fees to the board.

(2)

(A)

(i) A temporary license may be issued by the board for an applicant who has a current license from another state or territory. The temporary license allows the applicant to perform under qualified supervision the functions specified in §§ 63-11-202 and 63-11-203.

(ii) In order to receive a temporary license, the applicant must submit a completed application for a temporary license with the applicant's completed application for the desired license and all appropriate fees to the board.

(iii) Applications for temporary licenses for applicants currently licensed in another state or territory must be reviewed and approved or denied by a current member of the board designated by the board to perform this function.

(iv) If an applicant is issued a temporary license, then the license remains valid until the board grants or denies the license application.

(v) However, a person must not be issued more than one (1) temporary license, and a temporary license must not be valid for a period of more than one (1) year.

(B) Such a temporary license may be issued to allow an applicant to perform the functions specified in § 63-11-203 if the applicant has:

(i) Successfully completed an approved American Psychological Association academic course of study;

(ii) Previously taken and passed, according to this state's standards, the national licensing examination; and

(iii) A current license in another state or territory in good standing.

(C) Such a temporary license with designation as a health service provider may be issued to an applicant who meets the requirements of subdivision (e)(2)(B) and has:

(i) Successfully completed an American Psychological Association-approved internship; and

(ii) Had at least one (1) year of full-time postdoctoral experience delivering health services.

(D) Such a temporary license may be issued to allow an applicant to perform the functions specified in § 63-11-202 if the applicant has:

(i) Previously taken and passed, according to Tennessee standards, the national licensing examination; and

(ii) A current license in another state or territory in good standing.

(3) The board shall set the fee for all temporary licenses in an amount not to exceed one hundred dollars (\$100). Until such time as the board establishes a fee by rule, the fee is fifty dollars (\$50.00).

(f) This chapter does not expand or limit, in any way, the practice of psychological examiners as provided in § 63-11-202, senior psychological examiners as provided in § 63-11-202, or certified psychological assistants as provided in subdivision (c)(2) and § 63-11-207(b), and does not limit the use of the title "psychological examiner" for persons duly licensed as psychological examiners.

(g) Notwithstanding subsection (a), persons providing services in programs of the department of mental health and substance abuse services or the department of disability and aging either as employees or through contract agencies are not engaged in the practice of psychology and need not be licensed or certified under this chapter unless such persons provide services as a psychological examiner, psychologist, senior psychological examiner, certified psychological assistant, or certified psychological testing technician.

(h)

(1) A provisional license to practice as a psychologist with the designation as a health service provider may be issued by the board of examiners in psychology to an applicant who has successfully completed the academic and internship requirements of § 63-11-208(c)(1) and (d)(1) and who is to be engaged in a one-year postdoctoral experience required by § 63-11-208(d)(2)(A). The provisional licensure allows the applicant to perform the functions specified in §§ 63-11-203 and 63-11-208(d)(2)(B) under the qualified supervision of a psychologist acceptable to the board and who is licensed in this state with a designation as a health service provider in good standing.

(2) In order for a provisional license to remain valid, the licensee must take and pass the examination for professional practice in psychology (EPPP) at

the level set for licensure as a psychologist in this state within one (1) year of the issuance of the provisional license unless the applicant previously passed the EPPP at that level. A provisional license is revoked thirty (30) days after the date a provisional licensee receives notice of the licensee's second failure of the EPPP following issuance of the provisional license. During the thirty-day period prior to revocation of the provisional license, the licensee and the licensee's supervisor shall ensure the notification and transfer of care for the provisional licensee's clients.

(3) In order for the provisional license to remain valid, the licensee must take and pass the oral or jurisprudence examination, or both, developed by the board within two (2) years of the issuance of the provisional license. A provisional license is revoked thirty (30) days after the date a provisional licensee receives notice of the licensee's second failure of the oral or jurisprudence examination following issuance of the provisional license. During the thirty-day period prior to revocation of the provisional license, the licensee and the licensee's supervisor shall ensure the notification and transfer of care for the provisional licensee's clients.

(4) The board may consider petitions of the licensee regarding extraordinary circumstances that would necessitate delay in taking the written or oral examination.

63-11-207. License for psychological examiner, certified psychological assistant, and certified psychological testing technician.

(a) The board shall not issue new psychological examiner licenses. The discontinued issuance of psychological examiner licenses or the designation of senior psychological examiners in no way impairs or abridges the right of duly licensed psychological examiners to practice within the scope of their license or to renew their licensure status as required by law. Unless the person has obtained a license to

practice as a psychological examiner or senior psychological examiner, as issued by the board, it is unlawful for the person to practice. A person violates this chapter if the person practices as a certified psychological testing technician without first having obtained a proper certificate.

(b)

(1) A person seeking to practice as a certified psychological assistant in this state shall, before it is lawful for the person to practice as a certified psychological assistant, make application to the board, upon such form and in such manner as is adopted and prescribed by the board, including, but not limited to, the proper registration with the board of the supervisor of record for the certified psychological assistant. Unless the person obtains a proper certificate as issued by the board, it is unlawful for a person to practice, and if the person practices as a certified psychological assistant without first having obtained a proper certificate, then the person has violated this chapter.

(2) In addition to the requirements of § 63-11-107, a candidate for certification shall furnish the board with satisfactory evidence that the candidate:

(A) Is not considered by the board to be engaged in unethical practice; and

(B) Has had two (2) academic years of graduate training in psychology, including a master's degree, from an accredited educational institution recognized by the board as maintaining satisfactory standards, or in lieu thereof, such training and experience as the board considers equivalent thereto. Such graduate training must include specialized training in psychological assessment, test construction and measurement, the administration and interpretation of psychological testing, interviewing skills, and social history methods. The board may, at its discretion, consider equivalent training and experience when presented by qualified

applicants with a master's degree in psychology whose credentials differ slightly than those prescribed in this subdivision (b)(2)(B). However, all applicants must in their graduate training show evidence of supervised practicum experience in the administration and interpretation of psychological testing, assessment procedures, and social history and feedback interviewing techniques as determined by the board in order to fulfill educational requirements for certification.

(c)

(1) A person seeking to practice as a certified psychological testing technician in this state shall, before it is lawful for the person to practice as a certified psychological testing technician, make application to the board, upon such form and in such manner as is adopted and prescribed by the board, including, but not limited to, the proper registration with the board of the supervisor or supervisors of record for the certified psychological testing technician. Unless the person obtains a proper certificate as issued by the board, it is unlawful for the person to practice. A person violates this chapter if the person practices as a certified psychological testing technician without first having obtained a proper certificate.

(2) As used in subdivision (c)(1), "practice as a certified psychological testing technician" means the individual is duly authorized by the board to administer and score standardized psychological and neuropsychological tests and to observe and describe a client's test behavior and test responses. A certified psychological testing technician shall not select psychological or neuropsychological tests or versions of tests to be given, interpret test results, write test results, or give test feedback to clients.

(3) The services provided by a certified psychological testing technician must be provided under the supervision of a psychologist or senior psychological

examiner who is registered with the board as a supervisor of record to provide such supervision for the certified psychological testing technician. The certified psychological testing technician must be employed by, or contracted with, the same work setting as the supervising psychologist or senior psychological examiner who is registered with the board.

(4) In addition to the requirements of § 63-11-107, a candidate for certification shall furnish the board with satisfactory evidence that the candidate:

(A) Is not considered by the board to be engaged in unethical practice; and

(B) Proof of completion of a bachelor's degree or graduate degree, or proof of current enrollment in a graduate degree program, from a regionally accredited university, college, or professional school, in either psychology or education with a specialization in educational psychology, counseling psychology, or school psychology. The candidate's educational training must consist of a minimum of seventy-two (72) hours of total education and training relating to psychological or neuropsychological test administration and scoring that includes the following:

(i) At least twenty (20) hours of direct observation, including at least ten (10) hours of direct observation of a licensed psychologist or senior psychological examiner administering and scoring tests, and at least ten (10) hours of direct observation of either a licensed psychologist, senior psychological examiner, or certified psychological testing technician duly registered by the board administering and scoring tests;

(ii) At least forty (40) hours of administering and scoring tests in the presence of a licensed psychologist or senior psychological examiner; and

(iii) At least twelve (12) hours of additional education on topics related to best practices for the functions identified in subdivision (c)(2). Of those twelve (12) hours, at least three (3) hours must be specifically dedicated to law and ethics, and at least three (3) hours must be dedicated to multicultural competency. Additional topics must relate to growth, knowledge, and skill-based training that would enhance the candidate's professional skill in providing care for persons with medical, psychiatric, neurocognitive, and developmental conditions commonly assessed by psychologists and neuropsychologists.

(5) The education and training specified in subdivisions (c)(4)(B)(i)-(iii) may be obtained through any combination of the following:

(A) Participating in individual or group instruction by a licensed psychologist or senior psychological examiner;

(B) Engaging in independent learning directed by a licensed psychologist or senior psychological examiner;

(C) Completing graduate-level coursework at a regionally accredited university, college, or professional school; or

(D) Taking continuing education courses from organizations offering continuing education sponsorships.

(6) This chapter does not prevent a person engaged in gaining the experience, training, or education required by subdivision (c)(4)(B) from administering and scoring psychological and neuropsychological tests.

63-11-208. License for psychologist.

(a) A person wishing to obtain the right to practice as a psychologist in this state, who has not been licensed on or before the effective date of this act, shall, before it is lawful for the person to practice psychology in this state, make application to the board through the chair upon such form and in such manner as is adopted and prescribed by the board.

(b) Unless the person has obtained a license as prescribed in subsection (a), it is unlawful for the person to practice, and if the person practices psychology without first having obtained such a license, then the person has violated this chapter.

(c)

(1) In addition to the requirements of § 63-11-107, a candidate for such license shall furnish the board with satisfactory evidence that the candidate:

(A) Has received a doctorate in psychology from an accredited educational institution recognized by the board as maintaining satisfactory standards. After December 31, 1995, all such doctoral programs must be approved by the American Psychological Association and listed by the American Association of State Psychology Boards, or meet equivalent standards as are determined by the board;

(B) Is competent in psychology, as shown by passing such examinations, written or oral, or both, as the board deems necessary;

(C) Is not considered by the board to be engaged in unethical practice; and

(D) Has not within the preceding six (6) months failed an examination given by the board.

(2) The board may, at its discretion, consider equivalent training and experience when presented by qualified applicants with a doctoral degree in psychology whose credentials differ slightly than those prescribed in subdivisions

(c)(1)(A) and (d)(2)(A). In such cases, the applicant must petition the board for a waiver and specifically request the board utilize its discretionary authority.

(d) A licensed psychologist who provides or offers health services to the public must be designated by the board as a health service provider. In addition to the requirements for licensure as a psychologist prescribed in subsection (c), an applicant for designation as a health service provider must demonstrate to the board that the applicant:

(1) Has had at least a one-year internship at a site where health services are normally provided and that is part of an organized integrated training program. After December 30, 1995, all such internships must be approved by the American Psychological Association (APA) or listed by the Association of Psychological Post-Doctoral and Internship Centers (APPIC), or meet equivalent standards as are determined by the board; and

(2)

(A) Has successfully completed at least one (1) year postdoctoral experience delivering health services, supervised by a licensed psychologist designated by the board as a health service provider. If the one (1) year of postdoctoral experience was not in this state, then equivalent supervision must be documented to the satisfaction of the board; and

(B) For purposes of this chapter, "health service," in any variation, means the delivery of direct, preventive, assessment, and therapeutic intervention services to individuals whose growth, adjustment, or functioning is actually impaired or may be at risk of impairment.

(e) Notwithstanding subsection (d), the board may designate as a health service provider, with practice restriction to recipients in an intellectual disabilities' facility under the department of disability and aging, a person who meets all of the following criteria:

(1) The person holds a doctorate in psychology from a regionally accredited institution;

(2) The person is currently licensed as a psychologist in at least one (1) state;

(3) The person has academic training in intellectual disability and postdoctoral clinical experience in the field of intellectual disability of at least five (5) years' duration;

(4) The department submits to the board a description of the applicant's proposed functioning at the intellectual disability facility;

(5) The department certifies that:

(A) The applicant has training and experience adequate to perform the applicant's proposed functioning within the department;

(B) The department is unable to recruit otherwise qualified psychologists to provide psychological services; and

(C) This inability threatens the integrity of the department's services and the ability of the department to meet the needs of its clients; and

(6) A psychologist granted designation as a health service provider by the board under this subsection (e) shall surrender this designation upon termination of the person's employment with the department, which designation is, thereafter, null and void.

(f)

(1) Notwithstanding subsection (d), the board may designate as a health service provider, a person who has held a valid license or certificate in another state to practice psychology for at least ten (10) years within the last fifteen (15) years preceding the date of application and whose previous license or certification:

(A) Required training that is generally equivalent to the following licensing standards in this chapter:

- (i) A doctoral degree in psychology;
- (ii) One (1) year of internship in an organized integrated training program at a site delivering health services; and
- (iii) One (1) year of postdoctoral training delivering health services;

(B) Has never been the subject of disciplinary action; and

(C) Allowed the licensee to engage in practice as a health service provider in psychology in the other state.

(2) In order to effectuate the purposes of subdivision (f)(1), the board may promulgate emergency rules pursuant to § 4-5-208(a)(5). The rules must be promulgated in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

63-11-209. Examination of applicants for a license or certificate to practice psychology.

(a) Examination of applicants for a license or certificate to practice psychology may be made by the board at least once a year according to methods and in such subject fields as may be deemed by the board to be the most practical and expeditious to test the applicant's qualifications. Alternatively, the board may accept a nationally recognized test.

(b)

(1) Such examinations must include the basic psychological sciences.

(2) The board shall require the examinations to be written or oral, or both; provided, that in any written examination, the applicant is designated by a number instead of the applicant's name so that the applicant's identity is not

disclosed to the members of the board until the examination papers have been graded.

(3) The board shall grade the written examinations returned by the candidates and keep them for at least one (1) year.

(4) A candidate is held to have passed the examination upon the affirmative vote of three (3) or more members of the board.

(5) An unsuccessful candidate may, upon written request to the board, see the candidate's graded paper.

63-11-210. Certification without examination.

(a) The board may, at its discretion, grant a certificate without an assembled examination to a person residing or employed in the state who has not previously failed the appropriate examination given by the board and who, at the time of application, is licensed or certified by a similar board of another state whose standards, in the opinion of the board, are not lower than those required by this chapter, or who has been practicing psychology in another state and has qualifications not lower than those required by this chapter, and is able to satisfy to the board that to grant that person a license would be in the public interest, or who has been certified by the American Board of Examiners in Professional Psychology.

(b)

(1) Notwithstanding subsection (a), the board shall grant a license without an assembled written or oral examination to a person who at the time of application:

(A) Is licensed or certified in good standing with a similar board of another state;

(B) Has passed the National Examination for the Practice of Professional Psychology at a level greater than or equal to the level required in this state;

(C) Can show reasonable proof that the applicant has available employment in this state by means of letters or other reasonable proof; and

(D) Received a psychologist license after January 1, 1995, but before June 1, 1995.

(2) The board may permit a psychologist licensed in good standing in another state, who meets standards acceptable to the board, to perform the functions of §§ 63-11-203 and 63-11-208(d)(2)(B) and practice as a psychologist in this state without possessing a current license for a period of time, not to exceed twelve (12) days per year, for such purposes as special training or consultation, special evaluation, or intervention or serving as an expert witness.

(c) This section does not permit the regular, repetitive, or ongoing provision of psychological services, the supervision of psychological services, or the solicitation or advertisement of services to the general public, all of which are governed by the usual and customary processes of licensure for psychologists.

63-11-211. Confidential relations and privileged communications.

For the purpose of this chapter, the confidential relations and communications between licensed psychologist, licensed psychological examiner, licensed senior psychological examiner, certified psychological assistant, or certified psychological testing technician and client are placed upon the same basis as those provided by law between attorney and client. This chapter does not require such privileged communication to be disclosed.

63-11-212. Code of ethics.

The board of examiners shall, by rule, adopt a code of ethics to govern appropriate practices or behavior as referred to in §§ 63-11-109, 63-11-207, and 63-11-208.

63-11-213. Peer review – Immunity.

(a) As used in this section, "peer review committee" or "committee" means a committee, board, commission, or other entity constituted by a statewide psychological association or local psychological association for the purpose of receiving and evaluating professional acts of other practitioners of psychology as defined in this chapter.

(b) A practitioner of psychology who serves on a peer review committee or on any other committee is immune from liability with respect to an action taken in good faith and without malice as a member of such committee, board, commission, or other entity.

(c) Practitioners of psychology, individuals appointed to a committee as defined in subsection (a), and members of the boards of directors of facilities, as described in § 68-11-201, are immune from liability to a client, patient, individual, or organization for furnishing information, data, reports, or records to any such committee or for damages resulting from decisions, opinions, actions, or proceedings rendered, entered, or acted upon by such committee, undertaken or performed within the scope or functions of the duties of such committee, if made or taken in good faith and without malice and on the basis of facts reasonably known or reasonably believed to exist.

(d)

(1) All information, interviews, reports, statements, memoranda, or other data furnished to a peer review committee, associational board, or governing board, and any findings, conclusions, or recommendations resulting from the proceedings of such committees, associational boards, and governing boards are privileged.

(2) The records and proceedings are confidential and must be used only in the exercise of the proper functions of the committees, associational boards, or governing boards and not become public records nor be available for court subpoena or discovery proceedings.

(3) This subsection (d) does not apply to records, documents, or information otherwise available from original sources, and such records,

documents, or information are not immune from discovery or use in civil proceedings solely due to presentation to the committee.

63-11-214. Internships.

A person currently enrolled in an internship program satisfying the requirements of § 63-11-208(d)(1) and who completes the internship and all other degree requirements by December 31, 1992, must be designated as a health service provider without completing the one-year full-time supervised postdoctoral experience as provided in § 63-11-208(d)(2) if an application for such designation is submitted by July 1, 1993.

63-11-215. Health service provider designation – Criteria.

(a) For purposes of licensure renewal, a psychologist licensed under this chapter as a school, clinical, or counseling psychologist who was designated by the board as a health service provider by July 1, 1993, must continue to be renewed by the board with a designation of health service provider as long as the psychologist meets all other requirements for a renewal license.

(b) A psychologist licensed under this chapter other than as a school, clinical, or counseling psychologist who was designated by the board as a health service provider by July 1, 1993, must continue to be renewed by the board with a designation of health service provider as long as the psychologist meets all other requirements for licensure renewal.

63-11-216. Definitions for federal legislation and regulations and this code.

For purposes of federal legislation and regulations referencing qualifications with respect to the provision of psychological services and other titles in this code, unless the context otherwise requires, "licensed clinical psychologist," "licensed counseling psychologist," or "licensed school psychologist," in any variation, refers to a psychologist licensed under this chapter and designated as a health service provider.

63-11-217. Third-party reimbursement.

Community mental health centers, as defined in § 33-1-101; institutes or governmental agencies; doctoral clinical psychology students in university-operated training clinics; and those qualifying psychologists employing psychologists with temporary or provisional licenses are eligible to receive third-party reimbursement from a managed care plan or third-party payor for usual and customary professional services rendered by the employed psychologist with a provisional or temporary license.

63-11-218. Medical practitioners in free health clinics – Special volunteer license.

A psychologist licensed pursuant to this chapter under a special volunteer license who is a medical practitioner, as defined by § 63-1-201, engaged in practice at a free health clinic is not subject to license fees under this chapter. The board of examiners in psychology may issue a special volunteer license, as such license is defined in § 63-1-201, to qualified applicants without fee or charge. Such license is for a period of two (2) years and may be renewed on a biennial basis.

63-11-219. Notification of departure from community mental health center –

Patient records.

Psychologists, senior psychological examiners, and psychological examiners are not required to notify patients who were treated by them at community mental health centers, as defined in § 33-1-101, of their departure from such community mental health centers. Patient records must remain with the community mental health centers. The transfer of patients' records must be in compliance with standards set by state and federal law.

SECTION 28. Tennessee Code Annotated, Title 63, Chapter 11, is amended by deleting part 6.

SECTION 29. Tennessee Code Annotated, Title 63, Chapter 14, is amended by deleting the chapter.

SECTION 30. Tennessee Code Annotated, Title 63, Chapter 17, is amended by deleting part 2 and substituting:

63-17-201. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "Apprentice licensee applicant" means a person who is registered with the board, who is engaged in an approved licensing program, who is sponsored by a hearing instrument specialist duly licensed in this state, and who meets the qualifications of § 63-17-207;

(2) "Apprentice program" means sixty (60) classroom hours of prescribed coursework under the direct supervision of a hearing instrument specialist, with such coursework pertaining to all aspects of hearing instrument dispensing;

(3) "Board" means the board of communication disorders and sciences created by § 63-17-104;

(4) "Cerumen" means a wax-like secretion from glands in the external auditory canal;

(5) "Commissioner" means the commissioner of health;

(6) "Division" means the division of health related boards in the department of health;

(7) "Hearing instrument" means an instrument or device designed for or represented as aiding, improving, or correcting defective human hearing and any parts, attachments, or accessories of such an instrument or device;

(8) "Licensed hearing instrument specialist" means a person licensed by this part who has passed a board-approved psychometrically sound examination for hearing instrument specialists;

(9) "Otolaryngologist" means a physician specialist dedicated to the care of patients with disorders of the ears, nose, throat, and related structures of the head and neck, commonly referred to as ENTs;

(10) "Practice of dispensing and fitting hearing instruments" includes:

(A) The evaluation or measurement of the powers or range of human hearing by means of an audiometer for the consequent selection or adaptation for sale of hearing instruments intended to compensate for hearing loss, and the appropriate instructions, consultations, suggestions, recommendations, or opinions related to this practice;

(B) Making an impression of the ear, or an ear mold; and

(C) Cerumen management in the course of examining ears by a person who holds a certification of completion of a cerumen management course, as described in § 63-17-222; and

(11) "Sell" or "sale" includes a transfer of title or of the right to use by lease, bailment, or any other contract, either oral or written, except wholesale sales to distributors or dispensers.

63-17-202. Powers of the board.

In addition to the powers granted to the board in this chapter, the board shall:

- (1) Supervise and administer qualifying examinations to test the knowledge and proficiency of applicants for a license under this part;
- (2) License persons who apply to the board under this part and who are qualified to practice the fitting of hearing instruments;
- (3) Purchase and maintain or rent audiometric equipment and facilities necessary to carry out the examination of applicants for license under this part;
- (4) Issue and renew licenses under this part;
- (5) Deny, suspend, or revoke licenses pursuant to this part;
- (6) Appoint representatives to conduct or supervise the examination of applicants for license under this part;
- (7) Designate the time and place for examining applicants for license under this part;

- (8) Make and publish rules not inconsistent with the laws of this state that are necessary to carry out this part;
- (9) Require an annual calibration of audiometric equipment;
- (10) Provide all examinations required by this part or contract with organizations to conduct examinations;
- (11) Establish minimum requirements of test procedure and test equipment to be used in the fitting of hearing instruments and also the retention of records of all fittings;
- (12) Inspect the facilities or records, or both, of persons who practice the fitting and selling of hearing instruments, upon a showing of probable cause of a violation of this part;
- (13) Require lists, set standards, and approve programs for trainees as provided in § 63-17-206;
- (14) Promulgate rules requiring continuing education as a prerequisite for renewal of licenses to practice under this part; and
- (15) Employ a consultant to assist the board in its duties under this part.

63-17-203. Prohibited practice – Display of license.

(a) A person shall not engage in the sale of or practice of dispensing and fitting hearing instruments or display a sign or, in any other way, advertise or hold out to be a hearing instrument specialist unless direct hands-on testing and follow-up services are provided to persons to whom the hearing instrument specialist has dispensed or sold a hearing instrument, and such person holds a current, unsuspended, unrevoked license issued by the board as provided in this part.

(b) The license required by this section must be kept conspicuously posted in the specialist's principal office or place of business at all times.

63-17-204. Exemptions.

(a) This part does not prevent a person who is a physician licensed to practice by the board of medical examiners from treating or fitting hearing instruments to the human ear.

(b) It is lawful for a person, who meets the requirements of §§ 63-17-203 and 63-17-205 and who is listed as an apprentice licensee applicant with the board and who is under the direct supervision of a sponsoring licensed hearing instrument specialist, to engage in the practice of dispensing and fitting hearing instruments if the person does so under the direct supervision of the licensed hearing instrument specialist and with the licensed dispenser making final selection and fitting to the client. A licensed dispenser shall not supervise more than two (2) such apprentice licensee applicants or apprentice licensees combined at the same time.

(c) This part does not prevent a person who is an audiologist licensed by the board from practicing audiology, as defined in § 63-17-103, or prevent a person who is a speech-language pathologist licensed by the board from practicing speech-language pathology, as defined in § 63-17-103.

63-17-205. Applicant requirements.

(a) An applicant for a license must:

- (1) Be at least eighteen (18) years of age; and
- (2) Have an education equivalent to two (2) years of accredited college level coursework or national board for certification in hearing instrument sciences (NBC-HIS) board certification.

(b) Such applicant shall pay a nonrefundable application fee as set by the board no later than forty-five (45) days in advance of the next scheduled examination.

63-17-206. Apprentice license.

(a) Applicants who meet the requirements of § 63-17-205 with respect to age and education may apply for an apprentice license.

(b) The apprentice licensee functions under the direct supervision of the sponsoring licensed hearing instrument specialist for a period of at least three (3) months after submitting an application, and during such three-month period, the sponsoring licensed dispenser shall make the final selection and fitting of the hearing instrument.

(c) During the apprentice period, the apprentice licensee shall complete sixty (60) classroom hours of prescribed coursework.

(d) The apprentice license remains in effect no longer than one (1) year and cannot be renewed.

(e) Applicants for an apprentice license shall pay the board a nonrefundable fee as set by the board by rule.

63-17-207. Examination.

(a) Upon the payment of an examination fee as set by the board, an applicant for a license who is notified by the board that the applicant has fulfilled the requirements of § 63-17-205 shall appear at a time and place, and before such persons, as the board may designate to be examined by written and practical tests in order to demonstrate that the applicant is qualified to practice the fitting of hearing instruments.

(b) The board shall give at least one (1) examination of the type prescribed in this section each year and such additional examinations as the volume of applications may make appropriate.

63-17-208. Examination requirements.

(a) The examination described in § 63-17-207 must consist of:

(1) Tests of knowledge in the following areas as they pertain to the fitting of hearing instruments:

(A) Basic physics of sound;

(B) The human hearing mechanism, including the science of hearing and the causes and rehabilitation of abnormal hearing and hearing disorders; and

(C) Structure and function of hearing instruments;

(2) Tests of proficiency in the following techniques as they pertain to the fitting of hearing instruments:

(A) Pure tone audiometry, including air conduction testing and bone conduction testing;

(B) Live voice or recorded voice speech audiometry, including speech reception threshold testing and speech discrimination testing;

(C) Effective masking;

(D) Recording and evaluation of audiograms and speech audiometry to determine hearing instrument candidacy;

(E) Selection and adaption of hearing instruments and testing of hearing instruments;

(F) Taking earmold impressions; and

(G) Other skills as may be required for the fitting of hearing instruments; and

(3) Tests of knowledge of this part.

(b) The tests under this section do not include questions requiring a medical or surgical education.

63-17-209. License fee – License without examination.

(a) Upon payment of an initial license fee as determined by the board, the board shall issue a license to each applicant who passes the examination. An initial license expires as provided by rules promulgated by the board.

(b) The board may, in its discretion, license as a hearing instrument specialist, without examination, on the payment of a fee as set by the board, an applicant who is a

hearing instrument specialist, certified, registered, or licensed under the laws of another state or country, if the applicant's qualifications for certification, registration, or licensure meet the licensure requirements in force in this state upon the date of the specialist's application for certification, registration, or licensure in this state.

63-17-210. Notification by licensee of changes.

Each licensee shall notify the board of a change of address of the licensee's place of business within thirty (30) days of such change. Failure to give notice is deemed just cause for disciplinary action by the board.

63-17-211. Registry of licensees.

The board shall issue all licenses approved by the board and maintain a registry of all license holders. It is the duty of the board to review and approve the qualifications of applicants for certification or renewal as hearing instrument specialists.

63-17-212. Fees – Renewals.

(a)

(1) Each licensed hearing instrument specialist shall pay to the board a biennial renewal fee as set by the board, payable in advance, for the ensuing two (2) years.

(2) The board shall notify each licensee that such renewal is due. The failure of the licensee to receive such notice does not excuse the licensee's obligation to renew every two (2) years.

(b) When a licensed hearing instrument specialist fails to pay the license fee within sixty (60) days after it becomes due, as provided in this section, the license of the person is automatically revoked at the expiration of the sixty (60) days after the renewal fee was required, without further notice or hearing.

(c) A person whose license is automatically revoked as provided in § 63-1-107(c) may make application in writing to the council for the reinstatement of such license, and, upon good cause being shown, the council, in its discretion, may reinstate

such license upon payment of all past due renewal fees and upon further payment of a sum set by the council.

(d) A person licensed to practice by this part who has retired or may retire from such practice in this state on or after the effective date of this act is not required to register as provided by this part if the person files with the board an affidavit on a form to be furnished by the board, which affidavit states the date on which the person retired from such practice and such other facts as tend to verify the retirement as the board deems necessary. If the person thereafter reengages in such practice in this state, then the person must apply for registration with the board as provided by this part and meet other requirements as may be set by the board.

(e)

(1) Notwithstanding this part to the contrary, the division, with the approval of the commissioner, shall establish a system of license renewals at alternative intervals that allow for the distribution of the license workload as uniformly as is practicable throughout the calendar year. Licenses issued under the alternative method are valid for twenty-four (24) months and expire on the last day of the last month of the license period. However, during a transition period, or at any time thereafter when the board determines that the volume of work for any given interval is unduly burdensome or costly, either the licenses or renewals, or both of them, may be issued for terms of not less than six (6) months nor more than eighteen (18) months. The fee imposed for a license under the alternative interval method for a period of other than twenty-four (24) months must be proportionate to the annual fee and modified in no other manner, except that the proportional fee must be rounded off to the nearest twenty-five cents (25¢).

(2) A renewal application must not be accepted after the last day of the month following the license expiration date under the alternative method authorized in this subsection (e).

63-17-213. Fees relative to the practice of dispensing hearing instruments – Use of funds.

The board shall set the fees relating to the practice of dispensing hearing instruments in an amount sufficient to pay all of the expenses of the board that are directly attributable to the performance of its duties pursuant to this chapter.

Expenditures of the board for investigations and disciplinary actions against licensees of this part are the financial responsibility of the licensees who are regulated by this chapter. The division may maintain the existing separate accounts established for the board and former council for licensing hearing instrument specialists if the division combines the revenue collected such that the two (2) accounts may offset expenditures for the board. Funds remaining in these accounts at the end of any fiscal year do not revert to the general fund but remain available for expenditure in accordance with law.

63-17-214. Sales of hearing instruments.

(a) A person who engages in the practice of dispensing and fitting hearing instruments shall deliver to each person supplied with a hearing instrument, by the dispensing person's or at the dispensing person's order or direction, a bill of sale that contains each of the following:

(1) The signature of the licensed hearing instrument specialist, the address of the specialist's regular place of business, and the specialist's license number and license expiration date;

(2) The make and model of the hearing instrument supplied, and the amount charged for the hearing instrument and whether the hearing instrument is new, used, or rebuilt;

(3) A clear statement of the terms of sale, including the provisions required by § 63-17-215; and

(4) The name and address of the board and a statement that the board will receive complaints on any matter relating to the fitting and dispensing of hearing instruments.

(b) The sale of a hearing instrument is subject to the same conditions and provisions as are prescribed by the United States food and drug administration and the federal trade commission.

(c) A hearing aid that is sold, fitted, or dispensed to a consumer in this state must be clearly and permanently marked with:

(1) The name of the manufacturer or distributor or the model's name or number;

(2) The serial number; and

(3) The year of manufacture.

63-17-215. Returns.

(a) Within thirty (30) days of the date of delivery, a purchaser of a hearing instrument from a licensee is entitled to return the hearing instrument for any reason if such instrument is returned in satisfactory condition and such purchaser pays only reasonable charges for the hearing instrument and related services. Such return privileges apply only to a first-time purchaser of a hearing instrument.

(b) Charges to be imposed upon return of a hearing instrument as provided in subsection (a) must be clearly stated in the bill of sale.

(c) This section does not supersede any duly promulgated regulation issued by the federal trade commission.

63-17-216. Sponsor liability.

A person sponsoring an apprentice licensee or apprentice licensee applicant under this part is held responsible for the acts or omissions of the sponsor's apprentice

licensee or apprentice licensee applicant within the course and scope of the licensee's or applicant's employment, during the apprenticeship, as well as the sponsor's employees.

63-17-217. License denial, revocation, or suspension.

(a) Subject to the due process requirements of the Uniform Administrative Procedures Act, compiled in title 4, chapter 5, a person registered under this part may have such license denied, revoked, or suspended for a fixed period to be determined by the board for the following causes:

(1) Having a conviction of an offense involving moral turpitude. The record of such conviction or certified copy thereof from the clerk of the court where the conviction occurred or by the judge of such court is sufficient evidence to warrant revocation or suspension;

(2) Securing a license under this part through fraud or deceit;

(3) Engaging in unethical conduct or gross or repeated acts of ignorance or inefficiency in the conduct of the person's practice;

(4) Knowingly practicing while suffering with a contagious or infectious disease;

(5) Using a false name or alias in the practice of the profession; and

(6) Violating this part or the board's rules or orders.

(b)

(1) The board shall promulgate rules governing the assessment of costs against a licensee or other person found by the board to have violated this chapter. The costs assessed by the board may include only those costs directly related to the prosecution of the charges against the licensee or other person, including investigatory costs where appropriate. The board shall determine the appropriate amount of costs, if any, to be assessed in a contested case. These costs must be reasonable and proportionate in light of the violation committed by the licensee or other person.

(2)

(A) An elected officer of the board or a duly appointed or elected chair has the authority to administer oaths to witnesses. Upon probable cause being established, the board, by a vote of two-thirds (2/3) of the total membership of the board, may issue subpoenas for the attendance of witnesses and the production of documents and records.

(B) Service of a subpoena issued by the board must be made by the sheriff of the county of residence of the licensee or person upon whom the subpoena is served.

(C)

(i) A licensee or person served by subpoena has thirty (30) days to request in writing a hearing before the board for the sole purpose of making a special appearance to quash or modify the subpoena. The subpoena for attendance of the person or the production of documents and records is stayed until the board votes upon the request to quash or modify the subpoena. A majority vote of the total membership of the board is required to quash or modify a subpoena.

(ii) A motion to appeal from a decision by the board regarding a request to quash or modify a subpoena must be made to the chancery court in Davidson County within fifteen (15) days of such decision.

(D) If a witness fails or refuses to obey a subpoena issued by it, the board may make application to any court of record in this state within the jurisdiction of which the witness is found or resides, and the court has power to attach the body of the witness and compel the witness to appear before the board and give testimony or produce books, records, or papers

as ordered, and any failure to obey the court order may be punished by the court issuing the order as a civil contempt.

(E) Each witness who appears before the board by order of the board must receive for attendance the compensation provided by law for attendance of witnesses in a court of record, which must be paid from the funds of the board in the same manner as all other expenses of the board are paid.

63-17-218. Violations – Injunctions.

(a) A violation of this part is a Class B misdemeanor.

(b) The board may seek to enforce this part by petitioning a court of appropriate jurisdiction for an injunction to enjoin continuing violations of this part or by any other appropriate proceeding.

63-17-219. Persons licensed or in training up to July 1, 1995.

(a) A person who was a trainee or apprentice, or who had entered into a training program to become a licensed hearing aid dispenser, by July 1, 1995, is eligible to receive a license as a hearing instrument specialist if the person complies with the requirements of [former] chapter 15 of this title [repealed], as it existed on July 1, 1995.

(b) A person who was a licensed hearing aid dispenser on July 1, 1995, is eligible to receive a license as a hearing instrument specialist. A person who was a licensed hearing aid dispenser on July 1, 1995, who wishes to obtain a license as a hearing instrument specialist shall submit a request to the board to receive the appropriate license as a hearing instrument specialist. An additional fee shall not be imposed upon persons for the issuance of a license as a hearing instrument specialist pursuant to this subsection (b).

63-17-220. Cerumen management principles.

A licensed hearing instrument specialist shall comply with the following cerumen management principles:

(1) The indications for cerumen management for a licensed hearing instrument specialist include:

- (A) Enabling audiometric testing;
- (B) Making ear impressions;
- (C) Fitting hearing protection or prosthetic devices; and
- (D) Monitoring continuous use of hearing aids;

(2) The licensed hearing instrument specialist shall refer a patient who exhibits any of the following contraindications to cerumen removal for medical consultation or medical intervention to an otolaryngologist or a licensed physician:

- (A) An age less than twelve (12) years of age;
- (B) A perforated tympanic membrane;
- (C) History of pain, active drainage, or bleeding from the ear;
- (D) Evidence of congenital or traumatic deformity of the ear;
- (E) Ear surgery within the last six (6) months;
- (F) Tympanostomy tubes, such that irrigation should not be used;
- (G) A bleeding disorder;
- (H) Actual or suspected foreign body in the ear;
- (I) Stenosis or bony exostosis of the ear canal;
- (J) Cerumen impaction that totally occludes the ear canal;
- (K) Cerumen located medial to the cartilaginous external auditory canal; or

- (L) A tympanic membrane that the licensee is unable to see;

(3) In performing cerumen removal, a licensed hearing instrument specialist shall only remove cerumen lateral to the external auditory canal using the following instruments:

- (A) Cerumen loop;

(B) Cerumenolytic liquid;

(C) Irrigation, for patients with intact tympanic membranes and a closed mastoid cavity, no tympanostomy tubes, no recent ear surgery, and no recent dizziness; or

(D) Suction used lateral to the bony canal, only for patients with no recent surgery, intact tympanic membranes, and no clear otorrhea;

(4) If the patient, while undergoing cerumen management that did not present contraindications, complains of significant pain, exhibits uncontrolled bleeding or a laceration of the external auditory canal, or notices the acute onset of dizziness or vertigo or sudden hearing loss, then the licensed hearing instrument specialist must immediately stop the procedure and refer the patient to an otolaryngologist or a licensed physician;

(5) The licensed hearing instrument specialist shall maintain the following proper infection control practices:

(A) Universal health precautions;

(B) Decontamination;

(C) Cleaning, disinfection, and sterilization of multiple use equipment; and

(D) Universal precautions for prevention of the transmission of human immunodeficiency virus (HIV), hepatitis B virus, and other bloodborne pathogens, as defined by occupational safety and health standards promulgated pursuant to 29 CFR 1910;

(6) The licensed hearing instrument specialist who performs cerumen management shall maintain a case history for every patient and informed consent signed by the patient as part of the patient's records;

(7) The licensed hearing instrument specialist shall carry appropriate professional liability insurance before performing cerumen removal; and

(8) The licensed hearing instrument specialist shall not require patients to sign any form that eliminates liability if the patient is harmed.

63-17-221. Cerumen management course required – Rules.

(a) A licensed hearing instrument specialist who engages in cerumen management, pursuant to § 63-17-221, must have completed a cerumen management course approved by the International Hearing Society, the American Academy of Otolaryngology-Head and Neck Surgery, or another organization approved by the board.

The course must:

- (1) Be overseen by a physician, preferably an otolaryngologist;
- (2) Consist of at least six (6) hours of a participant practicing removing cerumen from an ear canal model using a variety of safe techniques; and
- (3) Result in a certificate of completion and attestation of competence signed by the overseeing physician.

(b) The board may promulgate rules to effectuate the requirements of the course outlined in this section only after consultation with the board of medical examiners, established by § 63-6-101. The rules must be promulgated in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

63-17-222. Transfer of rules, licensing and regulatory authority, to the board of communications disorders and sciences.

(a) The secretary of state shall transfer all rules promulgated by the [former] council for licensing hearing instrument specialists [repealed] and in effect on the effective date of this act to the board of communications disorders and sciences.

(b) Pursuant to the transfer of rules under subsection (a), the board of communications disorders and sciences is authorized to regulate hearing instrument specialists, including applying and enforcing the rules promulgated by the [former] council for licensing hearing instrument specialists, upon the effective date of this act.

SECTION 31. Tennessee Code Annotated, Title 63, Chapter 18, is amended by deleting the chapter.

SECTION 32. Tennessee Code Annotated, Title 63, Chapter 24, is amended by deleting the chapter.

SECTION 33. Tennessee Code Annotated, Title 63, is amended by deleting chapters 26, 27, 28, 30, and 31.

SECTION 34. Tennessee Code Annotated, Section 63-32-102(b), is amended by deleting the subsection and substituting:

(b) As used in this chapter, "provider" or "healthcare provider" means:

- (1) A physician, regulated pursuant to chapter 6 of this title;
- (2) An osteopathic physician, regulated pursuant to chapter 9 of this title;
- (3) A chiropractor, regulated pursuant to chapter 4 of this title;
- (4) A dentist, regulated pursuant to chapter 5 of this title;
- (5) A podiatrist, regulated pursuant to chapter 3 of this title;
- (6) An optometrist, regulated pursuant to chapter 8 of this title;
- (7) A dietitian or nutritionist, regulated pursuant to chapter 25 of this title;
- (8) A physician assistant, regulated pursuant to chapter 19 of this title;
- (9) A respiratory care practitioner, regulated pursuant to chapter 33, part 9 of this title;
- (10) A pharmacist, regulated pursuant to chapter 10 of this title;
- (11) An audiologist and speech pathology therapist, regulated pursuant to chapter 17 of this title;
- (12) A certified nurse practitioner, as such nurses are regulated pursuant to § 63-7-123;
- (13) A registered nurse anesthetist, regulated pursuant to chapter 7 of this title;
- (14) A social worker, regulated pursuant to chapter 23 of this title;

- (15) A psychologist, regulated pursuant to chapter 11 of this title;
- (16) A professional counselor, marital and family therapist, and clinical pastoral therapist, regulated pursuant to chapter 22 of this title;
- (17) A massage therapist, regulated pursuant to chapter 33, part 2 of this title;
- (18) Medical laboratory personnel, regulated pursuant to title 68, chapter 29;
- (19) Alcohol and drug abuse counselors, regulated pursuant to title 68, chapter 24;
- (20) Occupational therapists and physical therapists regulated pursuant to chapter 13 of this title;
- (21) A veterinarian, regulated by chapter 12 of this title; and
- (22) A nursing home administrator, regulated pursuant to chapter 16 of this title.

SECTION 35. Tennessee Code Annotated, Title 63, is amended by creating a new chapter 33, entitled "Commissioner Regulated Professions".

SECTION 36. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-101. Chapter definitions.

As used in this chapter:

- (1) "Commissioner" means the commissioner of health or a person to whom the commissioner has delegated authority granted to the commissioner in accordance with § 63-33-108;
- (2) "Department" means the department of health;
- (3) "Division" means the division of health related boards in the department of health, as created by § 68-1-101;

(4) "License" means a certificate, registration, or license created in this chapter; and

(5) "Licensee" means a person granted a certificate, registration, or license pursuant to this chapter.

63-33-102. Creation – Commissioner's authority and duties – Rules.

(a) There is created an agency of commissioner issued licenses.

(b) The commissioner is responsible for all administrative, fiscal, and clerical functions related to this chapter, which is accomplished through the division. All documents and fees applicants or licensees are required to submit to the commissioner by this chapter must be submitted to the division.

(c) For each of the licenses created by this chapter, the commissioner shall:

(1) Promulgate rules for the implementation of each part, including rules to establish:

(A) Requirements and procedures for the issuance of licenses and the renewal of licenses under each part;

(B) Standards of practice, ethics, advertising, and professional responsibility to govern the conduct of persons licensed under each part;

(C) Procedures for investigating reports of professional misconduct and unlawful acts by persons licensed under each part; and

(D) Criteria and procedures for certification by endorsement and reciprocity, waiver of examinations, and temporary certificates;

(2) Establish reasonable and necessary fees, including, but not limited to, licensure and licensure renewal that, in the aggregate, cover the cost of administering this chapter;

(3) Promulgate rules necessary for obtaining criminal background information from applicants prior to the issuance of a license; and

(4) Issue all licenses approved by the commissioner and maintain a list of persons holding licenses under each part, to be provided to any interested party upon written request.

(d) The commissioner shall also:

(1) Receive complaints and review issues related to complaints and disciplinary actions involving each person licensed under this chapter;

(2) Conduct investigations, as appropriate, of persons licensed pursuant to this chapter;

(3) Review the qualifications of licensure applicants and issue licenses to persons who meet the requirements for licensure under each part;

(4) Promulgate rules and criteria for licensee continuing education and for the renewal of licenses;

(5) Issue private letter rulings in accordance with § 63-1-174;

(6) Promulgate rules and perform any other action required to appropriately ensure the protection of the health, safety, and welfare of the public as related to a license created by this chapter;

(7) Collect or receive all fees, fines, and moneys owed pursuant to this chapter; and

(8) Utilize screening panels in the investigative and disciplinary process in the manner as follows:

(A) The screening panel must be made up of between one (1) and three (3) commissioner-appointed consultants who have authority to administer oaths to witnesses and deliberative privilege, and have the same immunity as provided by law for board members;

(B) The members of the screening panel are not subject to deposition or subpoena to testify regarding any matter or issue raised in a

contested case, criminal prosecution, or civil lawsuit that may result from or be incident to the screening panel matters processed before them;

(C) Documents or records produced at the screening panel are exempt from title 10, chapter 7, until there is a filing of a notice of charges and such documents or records form the basis for such filing of a notice of charges;

(D) The screening panel must be instructed as to the statutes and rules pertaining to the disciplinary action and the procedures to be followed by the panel, and provided Rule 31 of the Tennessee Supreme Court for guidance on the principles of alternative dispute resolution;

(E) The commissioner does not have the authority to compel a party to participate in a screening panel, and prejudice is not incurred if a party chooses not to participate or accept the offer of the screening panel; and

(F) The commissioner may promulgate rules regarding the screening panel process.

(e) The rules promulgated by the commissioner for the implementation of each part must comply with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(f) All moneys owed pursuant to this chapter, including the fees required by applicants or licensees and the costs, fines, and penalties, assessed for a violation of this chapter must be collected by the division. The division may maintain the existing separate accounts established for the boards and registries that formerly licensed the professions now licensed by the agency created by this chapter; provided, the division combines the revenue collected such that the division may use the revenue of each account to offset expenditures across the agency created by this chapter. Funds

remaining in these accounts at the end of any fiscal year do not revert to the general fund but remain available for expenditure in accordance with law.

(g) The commissioner shall use the department-employed investigators, inspectors, and agents, or use any other means necessary to bring about and maintain the rigid administration and enforcement of this chapter and all laws regulating the practice of the healings arts licensed by the commissioner.

63-33-103. License application.

(a) Applicants for a license created by this chapter shall submit an application on the form created by the commissioner and any required fees to the division.

(b) In addition to the specific requirements for licensure of each part of this chapter, the application must include evidence that the applicant meets the following requirements:

(1) The applicant has attained the age of majority;

(2) The applicant is of good moral character and conducts the applicant's professional activities in accordance with accepted professional and ethical standards;

(3) The applicant has not had a professional credential refused, revoked, suspended, or restricted and does not have a complaint, allegation, or investigation pending in any regulatory jurisdiction in the United States or in another country for unprofessional conduct, and has not committed an act that would constitute grounds for discipline of a license under § 63-33-106; and

(4) The applicant is either a citizen of the United States or a qualified alien, as defined in § 4-58-102.

(c) The division and applicant shall comply with the criminal background check requirements of § 63-1-116.

(d) All licensees shall comply with the requirements of § 63-1-109.

(e) Portions of an application involving an applicant's health condition, including mental health conditions and substance use disorders, and records involving an applicant's health condition, are confidential, privileged, and not public records subject to inspection by citizens of this state, as described in § 10-7-503.

(f)

(1) When the division receives a completed application for initial licensure from an applicant, the division shall, within sixty (60) days from the date the division receives the completed application:

(A) Render a decision on the application; or

(B) Inform the applicant of the need to appear before the commissioner or the commissioner's designee.

(2) As used in this subsection (f), "completed application" means an application that satisfies all statutory and board rule requirements.

63-33-104. Reciprocity.

(a) The commissioner may, at the commissioner's discretion, grant a license created by this chapter to a person residing or employed in this state who is certified or licensed by a similar entity in another state whose standards, as determined by the commissioner, are not less stringent than the standards required by the part creating the license.

(b) The commissioner may enter into a reciprocal agreement with another state or jurisdiction that certifies or licenses persons engaged in the practice of a profession licensed under this chapter if the commissioner determines that the other state has substantially similar or more stringent certification or licensure requirements than this state.

63-33-105. License renewal.

(a) A license issued under this chapter is valid for twenty-four (24) months from the date of issuance and is renewable biennially at the end of the birth month of the licensee.

(b) In addition to any requirements of this chapter creating the license, a licensee requesting renewal of a certificate shall provide the following to the division in advance of the expiration date of the license:

(1) A completed application on a form provided by the division and payment of the fees set by the commissioner; and

(2) A certified or sworn statement that the licensee:

(A) Has completed all the continuing education hours required for the licensee, whether required by statute or by rule, prior to the expiration date of the license; and

(B) Has not had a professional credential revoked, suspended, or restricted by a certification board, and does not have a complaint, allegation, or investigation pending in this state or in any regulatory jurisdiction for unethical or unprofessional conduct.

(c) If a licensee fails to submit an application for license renewal, to pay any fee required by the commissioner, or to submit a certified or sworn statement as set forth in subdivision (b)(2) prior to the expiration date of the license, then the license is automatically revoked by the commissioner without further notice or hearing, unless renewal is completed and all fees are paid prior to the expiration of sixty (60) days from the date of the expiration of the license.

(d) A person whose license is automatically revoked under this section may apply in writing to the division for reinstatement of the license. Upon the showing of good cause by the person and submitting documentation that all continuing education requirements have been satisfied, the commissioner may reinstate the license upon the payment of all fees set by the commissioner.

(e) Applicants and licensees must notify the division of the following within thirty (30) days of their occurrence:

- (1) A change of name, address, or other vital information;
- (2) The filing of any criminal charge or civil action against the applicant or licensee;
- (3) The initiation of any disciplinary charges, investigations, findings, or sanctions by a healthcare organization, federal or state agency, or other professional association against the applicant or licensee; and
- (4) Any other change in information provided by the applicant or licensee to the division.

(f) For the purpose of effecting service of process upon a licensee, the commissioner or the division may notify the licensee by certified mail, return receipt requested, at the address on file with the division.

(g) A person licensed or certified to practice by this chapter who has retired or may retire from such practice on or after the effective date of this act is not required to register as is required by this chapter if the person files with the division an affidavit on a form to be furnished by the division, stating the date on which the person retired from such practice and such other facts as tend to verify the retirement as the division deems necessary. If the person thereafter reengages in such practice in this state, then the person shall apply for licensure with the division as provided by this chapter and meet other requirements as may be set by the commissioner.

63-33-106. License denial, suspension, or revocation – Grounds – Examinations – Hearings.

- (a) The commissioner has the power to:
- (1) Deny an application for a license to any applicant who applies for the same through reciprocity or otherwise;
 - (2) Permanently or temporarily withhold issuance of a license;

(3) Issue a license contingent upon such circumstances or requirements as the commissioner may determine;

(4) Issue a conditional license that is limited or restricted for such time and in such manner as the commissioner may determine, including a limited license of temporary duration for applicants out of practice for an extended period of time;

(5) Issue discipline to an applicant or licensee, or on a previously issued license, including, but not limited to, any combination of, private censures and warnings, public reprimands, limits, restrictions, conditions, probation periods, suspension periods, require participation in a peer assistance program, and issue civil penalties for such time and in such manner as the commissioner may deem proper in the commissioner's discretion;

(6) Discipline a licensee who has been disciplined by another state of the United States for any acts or omissions that would constitute grounds for discipline licensed in this state, following the procedure in § 63-1-120; or

(7) Revoke, or permanently revoke, a license.

(b) The grounds upon which the commissioner may exercise the powers enumerated in subsection (a) include, but are not limited to:

(1) Engaging in unprofessional, dishonorable, or unethical conduct;

(2) Obtaining or attempting to obtain a license by making a false or misleading statement, failing to make a required statement, or engaging in fraud or deceit in any communication to the department;

(3) Violating or attempting to violate, directly or indirectly, or assisting in or abetting the violation of, or conspiring to violate, a criminal statute of this state, this part, a rule promulgated by the commissioner, or a lawful order of the commissioner issued pursuant to rules;

(4) Engaging in gross professional liability, or a pattern of continued or repeated professional liability, ignorance, negligence, or incompetence, in professional work;

(5) Being habitually intoxicated or personally misusing drugs, or using intoxicating liquors, narcotics, controlled substances, controlled substance analogues, or other drugs or stimulants, in such manner as to adversely affect the person's ability to safely practice in the person's profession;

(6) Being convicted of a felony, any offense under state or federal laws relative to drugs or the professional practice, an offense involving moral turpitude, or any offense for which the licensee or applicant is required to register as a sexual offender or violent sexual offender pursuant to the Tennessee Sexual Offender and Violent Sexual Offender Registration, Verification and Tracking Act of 2004, compiled in title 40, chapter 39, part 2;

(7) Engaging in professional practice when mentally or physically unable to do so safely; and

(8) Engaging in any other action deemed to be grounds for disciplinary action under rules promulgated or enforced by the commissioner.

(c) All proceedings for disciplinary action against a licensee under this chapter must be conducted in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(d) Assessment and collection of costs must follow the requirements and procedure contained in § 63-1-144.

(e) In enforcing this section, the commissioner shall, upon probable cause, have authority to compel an applicant or licensee to submit to a mental or physical examination, or both, by a designated committee of at least three (3) practicing physicians, including a psychiatrist where a question of mental condition is involved. The applicant or licensee may have an independent medical practitioner present during

such examination, and the applicant may have an independent physical or mental examination, which examination report must be filed with the commissioner for consideration. The committee shall submit a report of its findings to the commissioner, who must then hold a hearing governed by the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(f) Notwithstanding § 63-1-122, for purposes of an action before the commissioner for a license created and regulated under this chapter, in which the standard of care is at issue, qualified expert testimony on the standard of care is required. The standard of care for such actions is a statewide standard of minimal competency and practice. The provisions of title 29, chapter 26, and specifically § 29-26-115(a), concerning the locality rule, do not apply to disciplinary actions involving licenses created under this chapter. However, expert testimony is not required to prove a violation of unprofessional, dishonorable, or unethical conduct.

(g) A person whose license or certificate has been revoked by the commissioner is not eligible to apply for reinstatement earlier than one (1) year from the date of revocation.

63-33-107. Unlicensed practice.

(a) A person who engages in the practice of a profession licensed by the commissioner without a current, valid license, and a person who violates this chapter, commits a Class B misdemeanor. Each such engagement in unlicensed practice constitutes a separate offense.

(b) The commissioner may assess civil penalties against a person engaged in the practice of a profession licensed by the commissioner without a current, valid license for each separate offense. The commissioner may also refer the case to law enforcement.

(c) The commissioner may petition any circuit or chancery court having jurisdiction of a person within this state who is practicing without a license, to whom a

license has been denied, or whose license has been suspended or revoked, to enjoin such person from continuing to practice the healing arts, or any branch thereof, within this state. Jurisdiction is conferred upon the circuit and chancery courts of this state to hear and determine all such causes as equity causes and exercise full and complete jurisdiction in such injunctive proceedings.

(d) This section does not confer criminal jurisdiction upon a court not now possessing such criminal jurisdiction, nor does any such court, as an incident to the injunctive proceedings authorized in this section, have the power to assess the criminal penalties set out in this chapter.

63-33-108. Delegation of powers.

(a) The commissioner may delegate any of the powers granted to the commissioner in this chapter to any employee of the department. The commissioner may also appoint consultant members of any health-licensed profession to work for a per diem or hourly rate to review and evaluate licensure applications, disciplinary cases, licensee inquiries, serve on state or national advisory boards or compact commissions, and otherwise assist division staff as necessary to carry out this chapter.

(b) When appointing consultants pursuant to this section, the commissioner shall consider recommendations made by interested professional associations related to the licensure type at issue for the consultant's review.

63-33-109. Fees.

(a) By rule, the commissioner shall establish fees for all the license types created under this chapter. These fees include, but are not limited to:

- (1) Initial licensure application fee;
- (2) Examination fee;
- (3) Biennial licensure renewal fee;
- (4) Late renewal fee;
- (5) Licensure reinstatement fee;

(6) Duplicate decorative license fee; and

(7) Reciprocal licensure fee.

(b) The commissioner shall ensure that the fees collected by the division for the agency of commissioner issued licenses are sufficient in an amount to pay all the costs associated with the administration, monitoring, and enforcement for all the licenses created under this chapter.

63-33-110. Emergency rules.

To effectuate this chapter, the commissioner may promulgate emergency rules pursuant to § 4-5-208(a)(5).

63-33-111. Validity of prior licenses, certificates, and registrations.

All licenses, certificates, and registrations regulated under chapters 18, 24, 27, and 31, and valid as of June 30, 2026, must be transferred to the agency of commissioner issued licenses and remain in effect on their current renewal cycles under the authority of the commissioner pending the promulgation of any new rules on the issuance and renewal of such licenses, certificates, and registrations, as necessary.

63-33-112. Transfer of rules and regulatory authority to the commissioner.

(a) As of July 1, 2026, the secretary of state shall cause the following rules to be transferred to Tenn. Comp. R. & Regs. 1200-10:

(1) Tenn. Comp. R. & Regs. 0150-01;

(2) Tenn. Comp. R. & Regs. 0870-01 and 0870-02;

(3) Tenn. Comp. R. & Regs. 1330-01;

(4) Tenn. Comp. R. & Regs. 0880-14;

(5) Tenn. Comp. R. & Regs. 0880-12;

(6) All rules promulgated by the [former] professional art therapist advisory committee of the board of examiners in psychology [repealed]; and

(7) All rules promulgated by the [former] professional music therapy advisory committee of the board of examiners in psychology [repealed].

(b) The rules listed in subsection (a) must remain in effect pending the promulgation of any new rules on the same subject in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(c) Effective July 1, 2026, the commissioner has the authority to regulate the professions affected by the rules listed in subsection (a), including through use of those rules, until new rules on the same subject are promulgated.

SECTION 37. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-201. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "Compensation" means the payment, loan, advance, donation, contribution, deposit, or gift of money or anything of value;

(2) "Massage," "bodywork," and "somatic" mean the manipulation of the soft tissues of the body with the intention of positively affecting the health and well-being of the client;

(3) "Massage establishment" means a place of business held out to the public wherein massage is practiced; and

(4) "Massage therapist" means a person who practices massage for compensation and is licensed by the commissioner.

63-33-202. Practice of massage – Practice of reflexology.

(a) Persons or massage establishments engaged in massage for compensation must be licensed by the commissioner.

(b) It is unlawful to use the word "massage," or any other term that implies massage technique or method, when advertising a service by a person who is not licensed under this part or another chapter of state law.

(c) The practice of reflexology is not subject to the licensure requirements of this chapter. As used in this subsection (c), "reflexology" means the application of specific pressures to reflex points in the hands and feet only.

63-33-203. Documentation required from applicants.

(a)

(1) Except as provided in subdivision (a)(2) and in addition to the requirements of § 63-33-103, each applicant for licensure as a massage therapist shall provide documentation in the form required by the commissioner that the applicant:

(A) Successfully completed the curriculum or curricula of one (1) or more postsecondary academic institutions for massage, bodywork, or somatic therapy as defined by board rules, totaling six hundred fifty (650) hours or more, such institutions being approved by the commissioner pursuant to § 63-33-209, and either authorized by the Tennessee higher education commission, or its equivalent in other states, or approved, or under the governance of, the board of regents; and

(B) Received a passing score on a competency examination approved by the board.

(2)

(A) A person who has completed a program of study as required by subdivision (a)(1)(A) in a postsecondary academic institution located in this state and received a diploma or certificate prior to September 1, 2005, must be issued a license without completing the examination requirement in subdivision (a)(1)(B).

(B) A person who meets the requirements of subdivision (a)(2)(A) has until January 1, 2006, to apply for such a license.

(C) Persons licensed under this subdivision (a)(2) are not considered to have national certification and not hold themselves out to be nationally certified.

(b) In addition to the requirements of § 63-33-103, each applicant for licensure as a massage establishment shall provide documentation in the form required by the commissioner that each person engaged in massage at the massage establishment has provided the documentation required in subsection (a) and has been licensed as a massage therapist by the commissioner.

(c) Notwithstanding this part, an establishment license is not required for the office of a physician licensed under chapter 4, 6, or 9 of this title if a massage for compensation is provided within that office by a licensed massage therapist.

63-33-204. Inspections.

In order to effectuate this part, the commissioner may conduct an investigation of persons engaged in massage and massage establishments and inspect the license of practitioners and establishments for compliance. The refusal of a practitioner or establishment to permit inspections is grounds for revocation, suspension, or refusal to issue a license pursuant to this part.

63-33-205. Exempted persons.

This part does not affect, prevent, or restrict:

(1) Persons qualified and licensed to practice other health professions from engaging in the practice of massage when incidental to the practice of their profession;

(2) Persons qualified and licensed to practice as barbers or cosmetologists from engaging in the practice of massage when incidental to the practice of their profession;

(3) Students enrolled in an institution described in § 63-33-203(a)(1) or a public school of this state; provided, that the student does not hold the student's

self out as a licensed massage therapist and does not receive compensation for massage; and

(4) Massage therapists licensed in other states or countries when providing educational programs or services for a period of time not to exceed thirty (30) days within a calendar year.

63-33-206. Temporary license.

(a) The commissioner may, in the commissioner's discretion, grant a temporary license to a person who has been licensed, registered, or certified, in good standing, in another state, territory of the United States, or the District of Columbia, that meets all other requirements for reciprocity under this chapter and rules promulgated pursuant to this chapter, but which does not require successful passage of a competency examination approved by the commissioner.

(b) A person granted a temporary license pursuant to this section may work if the person is at all times working under the supervision of a person licensed pursuant to § 63-33-203.

(c)

(1) A temporary license issued pursuant to this section expires six (6) months after the date of issuance.

(2) A person granted a temporary license pursuant to this section may apply for one (1) extension of the person's temporary license for an additional six (6) months if:

(A)

(i) The person has been unable to take a competency exam approved by the commissioner within the initial temporary license period; and

(ii) Is registered to take an upcoming competency exam approved by the commissioner; or

(B) The person is still waiting for the results of the most recent competency exam approved by the commissioner.

63-33-207. Massage deemed medically therapeutic – Taxation.

Notwithstanding a law to the contrary, the act of a duly licensed massage therapist in performing a massage is deemed to be medically therapeutic in nature and not subject to the collection of any form of state or local taxation regulations not also imposed on other medically therapeutic activities.

63-33-208. Designation as "primary care provider" prohibited.

A person licensed by the commissioner under this part must not be referred to as a primary care provider or permitted to use such designation.

63-33-209. Powers and duties of commissioner – Passage rate of schools of massage therapy.

In addition to the powers and duties granted to or imposed by this chapter, the commissioner has the following powers and duties:

(1) To prescribe the minimum curricular and minimum standards for schools of massage therapy and for courses of training that prepare individuals for licensure under this part;

(2) To approve such schools and courses as meet the requirements of this part and the rules of the commissioner;

(3) To issue certificates of approval to such schools and courses that meet the requirements of this part and the rules of the commissioner;

(4) To conduct hearings for disciplinary action against schools or courses that fail to meet the minimum requirements of this part and the rules of the commissioner; and

(5) To annually publish passage rates for each school of massage therapy, based upon the results of each student's success in taking a national examination approved by the commissioner. Schools that fail to achieve an

overall passing rate of seventy percent (70%) are required to submit a remedial plan to be approved by the commissioner.

63-33-210. Alternative licensure process.

(a) The commissioner may issue a license to practice massage therapy to an applicant who:

(1) Meets the qualifications set forth in § 63-33-103; and

(2) Has been certified by the National Certification Board for Therapeutic Massage and Bodywork for the five-year period immediately preceding application for licensure and can submit documentation satisfactory to the commissioner that the applicant has engaged in the practice of massage therapy in another state for the five-year period immediately preceding application for licensure, and who either:

(A) Has met the qualifications set forth in § 63-33-203(a), but is unable, because the educational institution either was not state approved or is no longer in existence, to produce a transcript to document compliance; or

(B) Graduated from a qualified massage school or course prior to October 1, 1995.

(b) The commissioner may promulgate rules that are necessary to effectuate this section.

SECTION 38. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-301. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "Athletic injury" means an injury sustained by a person as a result of the person's participation in exercises, sports, games, or recreation requiring physical strength, agility, flexibility, range of motion, speed, or stamina, or

comparable athletic injury that prevents the person from participating in such activities; and

(2) "Athletic trainer" means a person with specific qualifications as set forth in this part, who, upon the advice, consent, and oral or written prescriptions or referrals of a physician licensed under this title, carries out the practice of prevention, recognition, evaluation, management, disposition, treatment, or rehabilitation of athletic injuries, and, in carrying out these functions, the athletic trainer may use physical modalities, such as heat, light, sound, cold, electricity, or mechanical devices related to prevention, recognition, evaluation, management, disposition, rehabilitation, and treatment; an athletic trainer shall practice only in those areas in which such athletic trainer is competent by reason of training or experience that can be substantiated by records or other evidence found acceptable by the board in the exercise of the commissioner's considered discretion.

63-33-302. Prohibited representations as an athletic trainer.

(a) A person shall not represent the person's self as, or claim to be, an athletic trainer or perform, for compensation, any of the activities of an athletic trainer without first obtaining a license under this part.

(b) This part does not prevent a person from serving as a student-trainer, or a similar position, if such service is not primarily for compensation and is carried out under the supervision of an athletic trainer duly licensed as provided in this part and a physician licensed under this title. Such supervision must be provided under the guidelines of the National Athletic Trainers' Association (NATA) Board of Certification, Inc., or approved by the commissioner.

63-33-303. Documentation required from applicants.

In addition to the requirements of § 63-33-103, an applicant for an athletic trainer license shall provide documentation in the form required by the commissioner that the applicant:

(1) Has met the athletic training curriculum requirements of a college or university approved by the commissioner; and

(2) Has satisfactorily completed all of the National Athletic Trainers' Association (NATA) Board of Certification, Inc., qualifications and is certified as an athletic trainer in good standing by the NATA Board of Certification, Inc., or approved by the commissioner.

63-33-304. Exempted persons.

(a) This part does not limit or prevent a person duly licensed or certified under the laws of this state from practicing the profession for which the person was licensed or certified.

(b) This part does not apply to a person engaging in the practice of athletic training while employed for such purpose by a postsecondary educational institution or professional athletic organization from another state that is temporarily present in this state for an athletic competition or exhibition.

(c) This part does not apply to physicians duly licensed under this title who employ or contract for services provided by persons in the physician's office assisting in the care of patients and rendered under the supervision, control, and responsibility of the licensed physician.

63-33-305. Civil penalties.

The commissioner has the authority to assess civil penalties commensurate with those assessable under § 63-1-134 against persons who have knowingly employed, contracted for, or otherwise utilized unlicensed persons in the practice of athletic training, with or without compensation.

63-33-306. Athletic trainers – Scope of practice.

(a) An athletic trainer may use dry needling, as defined in § 63-13-103, and physical modalities, such as heat, light, sound, cold, electricity, or mechanical devices, for the prevention, recognition, evaluation, management, disposition, rehabilitation, and treatment of athletic injuries.

(b) An athletic trainer shall only practice in those areas in which an athletic trainer is competent by reason of training or experience that can be substantiated by records or other evidence found acceptable by the board in the exercise of the commissioner's considered discretion.

(c) An athletic trainer shall obtain such training and certification in the practice of dry needling as the commissioner may require by rule. The practice of dry needling by an athletic trainer under this chapter does not constitute the practice of acupuncture under part 4 of this chapter.

(d) The commissioner is authorized to promulgate rules to effectuate this section. The rules must be promulgated in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

SECTION 39. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-401. Part definitions.

As used in this part, unless the context otherwise requires:

(1) "ACAOM" means the Accreditation Commission for Acupuncture and Oriental Medicine;

(2) "Acupuncture" means a form of health care developed from traditional and modern oriental medical concepts that employs oriental medical diagnosis and treatment and adjunctive therapies and diagnostic techniques for the promotion, maintenance, and restoration of health and the prevention of disease. "Adjunctive therapies" within the scope of "acupuncture" may include acupressure, cupping, thermal, and electrical treatment and the recommendation

of dietary guidelines and supplements and therapeutic exercise based on traditional oriental medical concepts;

(3) "ADS" means an acupuncture detoxification specialist trained in, and who performs only, the five-point auricular detoxification treatment;

(4) "NADA" means the National Acupuncture Detoxification Association;

(5) "NCCAOM" means the National Certification Commission for Acupuncture and Oriental Medicine; and

(6) "Practice of acupuncture" means the insertion of acupuncture needles and the application of moxibustion to specific areas of the human body based on oriental medical diagnosis as a primary mode of therapy.

63-33-402. Exempted persons – Unlawful practice of acupuncture – Limited licensure.

(a) This part does not apply to:

(1) Physicians licensed under chapter 6 or chapter 9 of this title, and does not prevent the practice of acupuncture by such physicians or to prevent such physicians from using the title "acupuncturist";

(2) Registered nurses who are nationally certified as holistic nurses and who have successfully completed an accredited education program in acupuncture; or

(3) Chiropractic physicians licensed under chapter 4 of this title, and does not prevent the practice of acupuncture by chiropractic physicians who have completed two hundred fifty (250) hours of an accredited acupuncture course and have passed the National Board of Chiropractic Examiners acupuncture exam.

(b) It is otherwise unlawful to practice acupuncture for compensation or gratuitously unless licensed under this part. This restriction does not apply to the following:

(1) Students practicing acupuncture under the supervision of a licensed acupuncturist as part of a course of study approved by the commissioner; or

(2) Individuals who do not otherwise possess the credentials required for the practice of acupuncture by this part, or rules promulgated under this part, by the commissioner are granted limited licensure as an ADS for the purpose of the treatment of alcoholism, substance abuse, or chemical dependency if they meet the following conditions:

(A) Provide documentation of successful completion of a commissioner-approved training program in auricular detoxification acupuncture that meets or exceeds standards of training set by NADA;

(B) Practice auricular detoxification treatment in a hospital, clinic, or treatment facility that provides comprehensive alcohol and substance abuse or chemical dependency services, including counseling, under the supervision of a licensed acupuncturist or medical director;

(C) Satisfy all appropriate ethical standards; and

(D) Limit their practice to the five-point auricular detoxification treatment.

63-33-403. Documentation required from applicants.

(a) In addition to the requirements of § 63-33-103, an applicant for licensure as an acupuncturist shall provide documentation in the form required by the commissioner that the applicant:

(1) Has successfully completed a three-year postsecondary training program or acupuncture college program that is ACAOM-accredited or in candidacy status or that meets ACAOM's standards;

(2) Has successfully completed an NCCAOM-approved clean needle technique course; and

(3) Either:

(A) Has current active status as a diplomate in acupuncture of the NCCAOM; or

(B) Has current state licensure in good standing by another state with substantially equivalent or higher standards.

(b) The commissioner shall issue a limited acupuncture license to an ADS who meets the requirements listed in § 63-33-402(b)(2).

63-33-404. License renewals.

In addition to the requirements of § 63-33-105, in order to renew a license, a person shall submit proof of current active NCCAOM certification in acupuncture or document compliance with § 63-33-403. To renew an ADS license, a person shall submit proof of current active practice in auricular detoxification treatment, as determined by the commissioner.

63-33-405. Health practices.

(a) All licensed individuals under this part shall use only presterilized, disposable needles in their administration of acupuncture treatments. The use of staples in the practice of acupuncture is prohibited.

(b) Health practices must include:

(1) Hands being washed with soap and water or other disinfectant before handling needles and between treatment of different patients;

(2) Skin in the area of penetration being thoroughly swabbed with alcohol or other germicidal solution before inserting needles; and

(3) Individuals passing a nationally recognized clean needle technique course before being allowed to practice acupuncture and related techniques.

63-33-406. Use of titles "licensed acupuncturist" or "ADS".

The titles "licensed acupuncturist" or "ADS" may be used by persons licensed under this part. A person who is not properly licensed to practice medicine or

osteopathic medicine shall not use licensure under this part to identify the person as a doctor or physician.

SECTION 40. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-501. Practice of art therapy.

(a) The practice of art therapy in this state is declared to affect the public health, safety, and welfare of citizens of this state.

(b) The commissioner is authorized to protect the public from:

(1) The practice of art therapy by unqualified persons; and

(2) Unprofessional, unethical, and harmful conduct by art therapy practitioners.

(c) A person shall not represent to be or function as a professional art therapist in this state unless the person holds a valid license issued by the commissioner. The commissioner shall also regulate the practice of art therapy.

63-33-502. Part definitions.

As used in this part:

(1) "Appraisal activities":

(A) Means selecting, administering, scoring, and interpreting art therapy instruments that are used to understand, measure, or facilitate an individual's normal human growth and development; and

(B) Does not include the use of projective techniques in the assessment of personality, nor the use of psychological or clinical tests designed to identify or classify abnormal or pathological human behavior, nor the use of individually administered intelligence tests;

(2) "Art therapy" means the integrated application of psychotherapeutic principles and methods with specialized training in visual art media, the neurobiological implications of art-making and the creative process, and art-

based assessment models to assist individuals, families, or groups to improve mental functioning, increase self-awareness and self-esteem, resolve conflicts and distress, and enhance social functioning;

(3) "Art therapy certified supervisor" means a person credentialed by the Art Therapy Credentials Board, Inc., to provide supervised clinical experience for an associate art therapist;

(4) "Associate art therapist" means a person holding a master's or doctoral degree in art therapy while engaged in the practice of professional art therapy under supervision as part of clinical training to fulfill the supervised experience requirement for becoming a licensed professional art therapist;

(5) "Certified art therapist" means a person holding national board certification (ATR-BC) from the Art Therapy Credentials Board, Inc.;

(6) "Credentialed art therapist" means a person holding credentials in good standing with the Art Therapy Credentials Board, Inc., as a registered art therapist (ATR), certified art therapist (ATR-BC), or art therapy certified supervisor;

(7) "Licensed professional art therapist" means a person who is licensed to practice professional art therapy in this state;

(8) "Practice of professional art therapy":

(A) Means rendering or offering to render to individuals, groups, organizations, or the general public any service as an independent or adjunctive therapist involving the application of art therapy principles, techniques, methods, or procedures; and

(B) Includes, but is not limited to, therapeutic interventions to facilitate alternative modes of receptive and expressive communication, development of treatment plans, appraisal activities, consulting and

referral activities, and prevention and treatment of mental and emotional conditions; and

(9) "Supervision" means the regular observation and oversight of the functions and activities of a graduate art therapy student as part of an internship or practicum experience, or an associate art therapist engaged in the practice of art therapy to fulfill the requirements for licensure under § 63-33-503.

63-33-503. Documentation required from applicants.

(a) In addition to the requirements of § 63-33-103, an applicant for licensure under this part shall provide documentation in the form required by the commissioner that the applicant has:

(1) Passed the board certification examination of the Art Therapy Credentials Board, Inc.; and

(2) Either:

(A) Received a master's degree or doctoral degree in art therapy from a program approved by the American Art Therapy Association or accredited by the Commission on Accreditation of Allied Health Education Programs at the time the degree was conferred, and has completed no less than one thousand (1,000) client contact hours of experience after the master's degree was conferred under the supervision of an art therapy certified supervisor, licensed professional art therapist, credentialed art therapist, or other licensed mental health professional approved by the commissioner. At least one-half (1/2) of the required supervision hours must be provided by an art therapy certified supervisor, licensed professional art therapist, or certified art therapist; or

(B) Received a master's degree or doctoral degree from a college or university that was accredited by a national or regional accrediting organization recognized by the Council for Higher Education Accreditation

in a program with a graduate-level course of study and clinical training that, as determined by the commissioner, is equivalent to an approved or accredited program in art therapy, and has completed not less than one thousand five hundred (1,500) hours of client contact experience after the qualifying degree was conferred under the supervision of an art therapy certified supervisor, licensed professional art therapist, credentialed art therapist, or other licensed mental health professional approved by the commissioner. At least one-half (1/2) of the required supervision hours must be provided by an art therapy certified supervisor, licensed professional art therapist, or certified art therapist.

(b) A person holding a national credential in good standing as a certified art therapist is deemed to have met the requirements of this section upon submitting an application and fee and providing evidence satisfying the requirements of § 63-33-103 in the form required by the commissioner.

63-33-504. Use of title "licensed professional art therapist" or "licensed art therapist."

(a) A person shall not use the title "licensed professional art therapist" or "licensed art therapist," or use an abbreviation to indicate or imply that the person is a licensed professional art therapist in this state unless the person holds a license issued by the commissioner under this part. This subsection (a) does not prohibit an associate art therapist from using the title "associate art therapist" in accordance with this part.

(b) A person shall not practice professional art therapy unless the person is:

(1) Licensed by the commissioner as a licensed professional art therapist;

(2) An associate art therapist, as long as the person uses the title of "associate art therapist" and does not represent themselves to the public as a licensed professional art therapist; or

(3) Exempt from licensure under § 63-33-506.

63-33-505. Authorized work settings.

(a) A licensed professional art therapist may practice independently for and with organizations that include, but are not limited to, addiction and rehabilitation centers, art studios and centers, behavioral health hospitals and outpatient settings, cancer treatment centers, community centers, community services boards, correctional facilities, disability services, eating disorder clinics, government agencies, medical hospitals and outpatient settings, military bases and veterans and family services, museums, nonprofits, nursing homes, physical rehabilitation, residential treatment centers, schools, senior living facilities, state programs, and wellness centers.

(b) Until the commissioner promulgates rules to establish the requirements for private outpatient independent practice by a licensed professional art therapist, a licensed professional art therapist shall only work in a private, outpatient independent setting if the patient is referred to the therapist by:

(1) A physician licensed under chapter 6 or 9 of this title;

(2) A psychologist licensed under chapter 11 of this title;

(3) A professional counselor licensed under chapter 22 of this title;

(4) A professional counselor designated as a mental health service provider under chapter 22 of this title;

(5) A marital and family therapist licensed under chapter 22 of this title;

(6) A pastoral counselor licensed under chapter 22 of this title;

(7) A licensed master's social worker licensed under chapter 23 of this title;

(8) A licensed clinical social worker licensed under chapter 23 of this title;

or

(9) Another advanced medical or mental health professional licensed under this title, as determined by the commissioner.

63-33-506. Authorized activities.

(a) This part does not:

(1) Require licensure or restrict the activities of a student enrolled in a graduate program in art therapy approved by the commissioner while practicing art therapy under qualified supervision and as part of an approved course of study; or

(2) Prohibit or restrict the practice of persons authorized to practice under this title who perform services consistent with the scope of practice of their profession and within the scope of their training and competence if the persons do not represent themselves to the public as a professional art therapist or as holding a license issued under this part.

(b) This part does not permit any method of treatment that involves the administration or prescription of drugs or the practice of medicine in this state or permits a person licensed as a professional art therapist to perform psychological testing.

(c) Subsection (b) does not prohibit a licensed professional art therapist from conducting appraisal activities. Consistent with each licensed professional art therapist's formal education and training, licensed professional art therapists may administer and utilize appropriate art therapy assessment instruments that measure or determine problems or dysfunctions within the context of human growth and development as part of the therapeutic process or in the development of a treatment plan.

SECTION 41. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-601. Practice of music therapy.

(a) The practice of music therapy in this state is declared to affect the public health, safety, and welfare of citizens of this state.

(b) The commissioner is authorized and given the duty to protect the public from:

(1) The practice of clinical music therapy by unqualified persons; and

(2) Unprofessional, unethical, and harmful conduct by music therapy practitioners.

(c) A person shall not represent to be or function as a professional music therapist in this state unless the person holds a valid license issued by the commissioner. The commissioner shall also regulate the practice of music therapy.

(d) This part does not restrict the non-clinical, non-therapeutic music intervention of the Tennessee person-centered music program administered by the Tennessee commission on aging and disability, created by § 52-8-102, and associated area agencies on aging and disability or their representatives.

63-33-602. Part definitions.

As used in this part:

(1) "Certified professional music therapist" means a person certified to practice music therapy under this part;

(2) "Music intervention" means the use of music in an individualized approach that includes music improvisation, receptive music listening, song writing, lyric discussion, music and imagery, singing, music performance, learning through music, music combined with other arts, music-assisted relaxation, music-based education, electronic music technology, adapted music intervention, and movement to music, that may be used in a clinical, therapeutic relationship or in a non-clinical, nontherapeutic manner; and

(3) "Music therapy":

(A) Means the clinical and evidence-based use of music interventions to accomplish individualized goals for people of all ages and ability levels within a therapeutic relationship by a board-certified music therapist; and

(B) Does not include the screening, diagnosis, or assessment of any physical, mental, or communication disorder.

63-33-603. Rules.

In addition to the authority granted by and requirements found in part 1 of this chapter, the rules promulgated by the commissioner for the implementation of this part must:

(1) Provide for the requirements for licensure as set forth in § 63-33-604;

and

(2) Take into consideration the standards of ethics and professional conduct of the code of ethics of the American Music Therapy Association and code of professional ethics of the Certification Board for Music Therapists in effect on January 1, 2025, or any successor organization.

63-33-604. Documentation required from applicants.

In addition to the requirements of § 63-33-103, the applicant shall provide documentation in the form required by the commissioner that the applicant:

(1) Holds a bachelor's degree or higher in music therapy, or its equivalent, including clinical training hours from a music therapy program within an accredited college or university;

(2) Has passed the examination for board certification offered by the Certification Board for Music Therapists, or any successor organization, or provides proof of being transitioned into board certification; and

(3) Is currently a board-certified music therapist.

63-33-605. Use of title "certified professional music therapist" or "certified music therapist."

(a) A person shall not use the title "certified professional music therapist," or "certified music therapist," or use any abbreviation to indicate that the person is certified as a professional music therapist in this state unless the person holds a certificate issued by the commissioner under this part.

(b) Subsection (a) does not prohibit:

(1) A person who is licensed, certified, or regulated under the laws of this state in another profession or occupation, including physicians, psychologists, psychoanalysts, registered nurses, marriage and family therapists, social workers, occupational therapists, professional or rehabilitation counselors, speech-language pathologists or audiologists, or personnel supervised by a licensed professional from performing work, including the use of music, that is incidental to the practice of the person's licensed, certified, or regulated profession or occupation, if the person does not represent themselves as a music therapist;

(2) A person whose training and national certification attests to the person's preparation and ability to practice their certified profession or occupation, if the person does not represent themselves as a music therapist;

(3) A practice of music therapy as an integral part of a program of study for students enrolled in an accredited music therapy program, if the student does not represent themselves as a music therapist;

(4) A person who practices music therapy under the supervision of a certified professional music therapist, if the person does not represent themselves as a music therapist; or

(5) The use of music intervention in a non-clinical, non-therapeutic manner by persons who do not represent themselves as music therapists or through a program that is not represented to be a music therapy program, including the Tennessee person-centered music program administered by the Tennessee commission on aging and disability, created by § 52-8-102.

63-33-606. Authorized work settings – Required reviews.

(a) A certified professional music therapist may practice independently for and with organizations that include, but are not limited to, addiction and rehabilitation centers, music studios and centers, behavioral health hospitals and outpatient settings,

cancer treatment centers, community centers, community services boards, correctional facilities, disability services, eating disorder clinics, government agencies, medical hospitals and outpatient settings, military bases and veterans and family services, museums, nonprofits, nursing homes, physical rehabilitation, residential treatment centers, schools, senior living facilities, state programs, and wellness centers.

(b)

(1)

(A) Before a certified professional music therapist provides music therapy services to a client for an identified clinical or developmental need, the certified professional music therapist shall review the client's diagnosis, treatment needs, and treatment plan with the healthcare providers involved in the client's care.

(B) During the provision of music therapy services to a client, the certified professional music therapist shall collaborate, as applicable, with the client's treatment team, including the client's physician, psychologist, licensed clinical social worker, or other mental health professional. During the provision of music therapy services to a client with a communication disorder, the certified professional music therapist shall collaborate and discuss the music therapy treatment plan with the client's audiologist or speech-language pathologist so that a music therapist may work with the client and address communication skills.

(2) Before a certified professional music therapist provides music therapy services to a student for an identified educational need in a special education setting, the certified professional music therapist shall review the student's diagnosis, treatment needs, and treatment plan with the individualized family service plan or individualized education program team.

(3) When providing educational or healthcare services, a certified professional music therapist shall not replace the services provided by an audiologist or a speech-language pathologist. Unless authorized to practice speech-language pathology, music therapists shall not evaluate, examine, or provide instruction or counsel on speech, language, communication, and swallowing disorders and conditions. A person certified as a professional music therapist shall not represent to the public that the person is authorized to treat a communication disorder. This subdivision (b)(3) does not prohibit a person, certified as a professional music therapist from representing to the public that the person is qualified to work with clients with communication disorders and to address communication skills.

63-33-607. Authorized activities.

In addition to any other authority granted under this part, certified professional music therapists may:

(1) Accept referrals for music therapy services from medical, developmental, mental health, or education professionals; family members; clients; caregivers; or others involved with and authorized to provide client services;

(2) Conduct a music therapy assessment of a client to determine if treatment is indicated. If treatment is indicated, then the certified professional music therapist may collect systematic, comprehensive, and accurate information to determine the appropriateness and type of music therapy services to provide for the client;

(3) Develop an individualized music therapy treatment plan for the client that is based upon the results of the music therapy assessment. The music therapy treatment plan must include individualized goals and objectives that focus on the assessed needs and strengths of the client and specify the music

therapy approaches and interventions to be used to address these goals and objectives;

(4) Implement an individualized music therapy treatment plan that is consistent with any other developmental, rehabilitative, habilitative, medical, mental health, preventive, wellness care, or educational services being provided to the client;

(5) Evaluate a client's response to music therapy and the music therapy treatment plan, documenting change and progress, and suggesting modifications, as appropriate;

(6) Develop a plan for determining when the provision of music therapy services is no longer needed in collaboration with the client, physician, or other provider of health care or education of the client, family members of the client, and any other appropriate person upon whom the client relies for support;

(7) Minimize barriers to ensure that the client receives music therapy services in the least restrictive environment;

(8) Collaborate with and educate the client and the family, caregiver of the client, or any other appropriate person regarding the needs of the client that are being addressed in music therapy and the manner in which the music therapy treatment addresses those needs; and

(9) Utilize their appropriate knowledge and skills to practice, which include the use of research, reasoning, and problem-solving skills in determining appropriate actions in the context of each clinical setting.

63-33-608. Psychological practice.

This part does not permit a certified professional music therapist to engage in a psychological practice as described in chapter 11, part 2 of this title.

SECTION 42. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-801. Part definitions – Setting for practice of polysomnography.

(a) As used in this part, unless the context otherwise requires:

(1) "Direct supervision" means that the polysomnographic technologist providing supervision must be present in the area where the polysomnographic procedure is being performed and immediately available to furnish assistance and direction throughout the performance of the procedure;

(2) "General supervision" means that the polysomnographic procedure is provided under a physician's overall direction and control, but the physician's presence is not required during the performance of the procedure;

(3) "Polysomnographic student" means a person who is enrolled in an educational program that is accredited by the Commission on Accreditation of Allied Health Education Programs (CAAHEP), as provided in § 63-33-803(a)(1), and who may provide sleep-related services under the direct supervision of a polysomnographic technologist as a part of the person's educational program;

(4) "Polysomnographic technician" means a person who has graduated from an accredited educational program described in § 63-33-803(a)(1) but has not yet passed the national certifying examination given by the Board of Registered Polysomnographic Technologists, who has obtained a temporary permit from the commissioner, and who may provide sleep-related services under the general supervision of a licensed physician;

(5) "Polysomnographic technologist" means a person who is credentialed by the Board of Registered Polysomnographic Technologists and is licensed by the commissioner to engage in the practice of polysomnography under the general supervision of a licensed physician;

(6) "Polysomnographic trainee" means a person who is enrolled in an accredited sleep technologist education program (A-STEP) that is accredited by the American Academy of Sleep Medicine and who may provide sleep-related

services under the direct supervision of a polysomnographic technologist as a part of the person's educational program;

(7) "Practice of polysomnography" means the staging and scoring of sleep by continuous and simultaneous monitoring of the stages of sleep and wake through use of an electroencephalogram (EEG), an electrooculogram (EOG), and a submental electromyogram (EMG), in conjunction with the recording and monitoring of other physiological variables, and the assignment of values for duration, frequency, and type of event to each stage of sleep in which the event occurred. The following tasks are considered a part of the practice of polysomnography only when performed as part of the staging and scoring of sleep under the general supervision of a licensed physician:

(A) Monitoring and recording physiologic data during the evaluation of sleep-related disorders, including sleep-related respiratory disturbances, by applying the following techniques, equipment, and procedures:

(i) Continuous or bilevel positive airway pressure titration on spontaneously breathing patients using a mask or oral appliance when the mask or oral appliance does not extend into the trachea or attach to an artificial airway;

(ii) Supplemental low flow oxygen therapy of less than six (6) liters per minute, utilizing nasal cannula or continuous or bilevel positive airway pressure during a polysomnogram;

(iii) Capnography during a polysomnogram;

(iv) Cardiopulmonary resuscitation;

(v) Pulse oximetry;

(vi) Gastroesophageal pH monitoring;

(vii) Esophageal pressure monitoring;

(viii) Sleep staging, including surface electroencephalography, surface electrooculography, and surface submental electromyography;

(ix) Surface electromyography;

(x) Electrocardiography;

(xi) Respiratory effort monitoring, including thoracic and abdominal movement;

(xii) Plethysmography blood flow monitoring;

(xiii) Snore monitoring;

(xiv) Audio or video monitoring;

(xv) Body movement monitoring;

(xvi) Nocturnal penile tumescence monitoring;

(xvii) Nasal and oral airflow monitoring;

(xviii) Body temperature monitoring; and

(xiv) Monitoring the effects that a mask or oral appliance used to treat sleep disorders has on sleep patterns; however, the mask or oral appliance must not extend into the trachea or attach to an artificial airway;

(B) Observing and monitoring physical signs and symptoms, general behavior, and general physical response to polysomnographic evaluation and determining whether initiation, modification, or discontinuation of a treatment regimen is warranted;

(C) Analyzing and scoring data collected during the monitoring described in subdivisions (a)(7)(A) and (B) for the purpose of assisting a licensed physician in the diagnosis and treatment of sleep and wake disorders that result from developmental defects, the aging process, physical injury, disease, or actual or anticipated somatic dysfunction;

(D) Implementation of a written or verbal order from a licensed physician that requires the practice of polysomnography; and

(E) Education of a patient regarding the treatment regimen that assists the patient in improving the patient's sleep; and

(8) "Sleep-related services" means acts performed by polysomnographic technicians, polysomnographic trainees, polysomnographic students, and other persons permitted to perform those services under this part, in a setting described in subsection (c) that would be considered the practice of polysomnography if performed by a polysomnographic technologist.

(b) A licensed dentist shall make or direct the making and use of any oral appliance used to treat sleep disordered breathing and shall evaluate the structures of the patient's oral and maxillofacial region for purposes of fitting the appliance.

(c) The practice of polysomnography must take place only in a hospital, a stand-alone sleep laboratory or sleep center, or in the patient's home in accordance with a physician's order; however, the scoring of data and the education of patients may take place in settings other than in a sleep laboratory, sleep center, or the patient's home.

63-33-802. Exemptions.

(a) This part does not limit or restrict a healthcare practitioner licensed under this title from engaging in the full scope of practice of the person's profession.

(b) This part does not apply to diagnostic electroencephalograms conducted in accordance with the guidelines of the American Clinical Neurophysiology Society.

63-33-803. Documentation required from applicants – License renewals.

(a) In addition to the requirements of § 63-33-103, an applicant for licensure under this part shall provide documentation in the form required by the commissioner that the applicant:

(1) Meets one (1) of the following educational requirements:

(A) Graduation from a polysomnographic educational program that is accredited by the Commission on Accreditation of Allied Health Education Programs;

(B) Graduation from a respiratory care educational program that is accredited by the Commission on Accreditation of Allied Health Education Programs and completion of the curriculum for a polysomnography certificate established and accredited by the Committee on Accreditation for Respiratory Care of the Commission on Accreditation of Allied Health Education Programs;

(C) Graduation from an electro neurodiagnostic technologist educational program with a polysomnographic technology track that is accredited by the Commission on Accreditation of Allied Health Education Programs; or

(D) Successful completion of an accredited sleep technologist educational program (A-STEP) that is accredited by the American Academy of Sleep Medicine;

(2) Has passed the national certifying examination given by the Board of Registered Polysomnographic Technologists;

(3) Is credentialed by the Board of Registered Polysomnographic Technologists; and

(4) Has met any additional educational or clinical requirements established by the commissioner.

(b) A person who was engaged in the practice of polysomnography on July 1, 2007, is eligible for licensure under this part without meeting the educational requirement of subdivision (a)(1) if the person meets the requirements of subdivisions (a)(2)-(4).

(c) To be eligible for renewal of a license to engage in the practice of polysomnography, a polysomnographic technologist must continue to be credentialed by the Board of Registered Polysomnographic Technologists.

63-33-804. Unlicensed persons authorized to provide sleep-related services.

(a) The following persons may provide sleep-related services without being licensed as a polysomnographic technologist under this part:

(1) A polysomnographic technician who provides sleep-related services under the general supervision of a licensed physician for a period of up to one (1) year from the date of the person's graduation from one (1) of the accredited programs described in § 63-33-803(a)(1). The commissioner may, in the commissioner's sole discretion, grant a one-time extension of up to three (3) months beyond this one-year period;

(2) A polysomnographic trainee who provides sleep-related services under the direct supervision of a polysomnographic technologist as a part of the person's educational program while actively enrolled in an accredited sleep technologist educational program (A-STEP) that is accredited by the American Academy of Sleep Medicine;

(3) A polysomnographic student who provides sleep-related services under the direct supervision of a polysomnographic technologist as a part of the person's educational program while actively enrolled in a polysomnographic educational program that is accredited by the Commission on Accreditation of Allied Health Education Programs (CAAHEP);

(4) A person who is credentialed in one (1) of the health-related fields accepted by the Board of Registered Polysomnographic Technologists who provides sleep-related services under the direct supervision of a polysomnographic technologist, for a period of up to one (1) year, while obtaining

the clinical experience necessary to be eligible to sit for the examination given by the Board of Registered Polysomnographic Technologists; and

(5) Respiratory therapists who provide polysomnography services after being credentialed as a registered polysomnographic technologist by the Board of Registered Polysomnographic Technologists, or as a sleep disorders specialist by the National Board for Respiratory Care, or have undergone a standardized, uniform mechanism to document competency in polysomnography as approved by the commissioner. Documentation of such a credential or of passage of this commissioner-approved mechanism must be made available at the request of the commissioner. Respiratory therapists are not required to have a second license as a polysomnographic technologist.

(b) Before providing sleep-related services, a polysomnographic technician shall obtain a temporary permit from the commissioner. While providing sleep-related services, the technician shall wear a badge that appropriately identifies the person as a polysomnographic technician.

(c) Before providing sleep-related services, a polysomnographic trainee shall give notice to the commissioner that the trainee is enrolled in an A-STEP educational program accredited by the American Academy of Sleep Medicine. Trainees shall wear a badge that appropriately identifies the trainee as a polysomnographic trainee while providing such services.

(d) Before providing sleep-related services, a person who is obtaining clinical experience pursuant to subdivision (a)(4) shall give notice to the commissioner that the person is working under the direct supervision of a polysomnographic technologist in order to gain the experience to be eligible to sit for the examination given by the Board of Registered Polysomnographic Technologists. The person shall wear a badge that appropriately identifies the person while providing such services.

(e) Polysomnographic students must not receive compensation for sleep-related services they provide and wear badges that appropriately identify them as students.

63-33-805. Use of title "Polysomnographic technologist."

A person who is licensed to engage in the practice of polysomnography in this state has the right to use the title "polysomnographic technologist" or the abbreviation "PSGP." Any other person shall not use that title or abbreviation, or any other words or letters, indicating that the person is a polysomnographic technologist.

SECTION 43. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-901. Part definitions – Authorized settings for the practice of respiratory care – Exempted persons.

(a) As used in this part, unless the context otherwise requires:

(1) "Certified respiratory therapist" means a person licensed under this part who has obtained and maintains the credential "certified respiratory therapist" (CRT) or the credential "certified respiratory therapy technician" (CRTT) from the National Board for Respiratory Care;

(2) "Practice of respiratory care":

(A)

(i) Means:

(a) Under the supervision, control, and responsibility of a licensed physician, the therapy, management, education and instruction, rehabilitation, diagnostic testing evaluation, and care of patients with deficiencies and abnormalities that affect the cardiorespiratory system and associated aspects of other system functions; or

(b) Under the supervision, control, and responsibility of a licensed physician, the performance of cardiorespiratory research, cardiorespiratory health promotion and disease prevention, and community wellness and education programs;

(ii) Includes, under the supervision, control, and responsibility of a licensed physician:

(a) The administration of pharmacologic agents and medical gasses necessary to diagnose, implement treatment, promote disease prevention, and provide rehabilitation to the cardiorespiratory system; however, a respiratory care practitioner shall not prescribe controlled substances of any kind;

(b) The performance of specific diagnostic and testing techniques employed in the medical management of patients to assist in diagnosis, patient and family education, monitoring, treatment, maintenance of patient health status, and cardiorespiratory research, including, but not limited to, pulmonary function testing, hemodynamic and physiologic monitoring and diagnosis of cardiac function, and specimen collection and analysis;

(c) The establishment and maintenance of the airways, bronchopulmonary hygiene, and cardiopulmonary resuscitation, and cardiac and respiratory life support; and

(d) The receipt and implementation of written and verbal physician orders and the development and implementation of care plans and protocols;

(3) "Registered respiratory therapist" means a person licensed under this part who has obtained and maintains the credential "registered respiratory therapist" (RRT) from the National Board for Respiratory Care;

(4) "Respiratory assistant" or "assistant" means a person certified as such pursuant to this part, who has:

(A) Received on-the-job training equivalent to at least one thousand (1,000) hours of clinical training in respiratory care, during an eighteen-consecutive-month period that was received under the supervision of:

- (i) A registered respiratory therapist;
- (ii) A certified respiratory therapist; or
- (iii) A licensed physician experienced in respiratory care;

and

(B) Passed a commissioner-approved examination for respiratory assistants. The National Board for Respiratory Care (NBRC) entry level examination, as provided to the board by the NBRC, by contract, is the examination accepted by the commissioner. This examination does not grant the respiratory assistant the NBRC credential. In addition, the commissioner may write the commissioner's own state board examination or contract with other national testing organizations; and

(5) "Respiratory care practitioner" means a registered respiratory therapist, a certified respiratory therapist, or a respiratory assistant licensed under this part.

(b) The practice of respiratory care may be performed in an inpatient or outpatient setting, clinic, hospital, nursing home facility, private dwelling, or other place deemed appropriate or necessary.

(c) Registered respiratory therapists, certified respiratory therapists, and respiratory assistants who have been issued a license or certificate by the commissioner, pursuant to the authority of § 63-33-902, are exempt from any licensing, certification, or permit requirements of the Tennessee Medical Laboratory Act, compiled in title 68, chapter 29, for medical laboratory personnel or medical laboratory trainees insofar as they engage in performing blood gases and PH analyses.

63-33-902. Licenses required – Documentation required from applicants – License upgrades.

(a) In order to safeguard life, health, and property and promote public welfare by requiring that only properly qualified persons practice respiratory care in this state, a person practicing respiratory care must be licensed as provided in this part, and it is unlawful for any person to practice or offer to practice respiratory care unless the person has been duly licensed under this part, except as provided in this section.

(b)

(1) In addition to the requirements of § 63-33-103, an applicant for licensure as a registered respiratory therapist shall provide documentation in the form required by the commissioner that the applicant has successfully completed a respiratory care educational program accredited by the Commission on Accreditation of Allied Health Education Programs (CAAHEP) in collaboration with the Committee on Accreditation for Respiratory Care (CoARC), or their successor organizations, and has successfully passed the National Board for Respiratory Care registry examination for advanced respiratory therapists.

(2) In addition to the requirements of § 63-33-103, an applicant for licensure as a certified respiratory therapist must provide documentation in the form required by the commissioner that the applicant has successfully completed a respiratory care educational program accredited by the CAAHEP in collaboration with the CoARC, or their successor organizations, and has

successfully passed the National Board for Respiratory Care certification examination for entry level respiratory therapists.

(3) After January 1, 2000, a person must not be licensed as a respiratory assistant if that person has not been previously licensed as a respiratory assistant in this state. However, persons who have been licensed as respiratory assistants may continue to renew their licenses indefinitely so long as they meet all the requirements for renewal.

(c)

(1) A person licensed to practice as a certified respiratory therapist may upgrade the license to registered respiratory therapist if the person furnishes evidence satisfactory to the commissioner that the person meets all the requirements to be licensed as a registered respiratory therapist.

(2) A person licensed to practice as a respiratory assistant may upgrade the license to certified respiratory therapist or registered respiratory therapist if the person furnishes evidence satisfactory to the commissioner that the person meets all the requirements to be licensed as a certified respiratory therapist or registered respiratory therapist.

(3) A person must not hold more than one (1) license to practice respiratory care in this state at the same time.

63-33-903. Knowledge and skills required of registered respiratory therapists – Independent clinical judgment authorized.

(a) The knowledge and skills for performing the functions of a registered respiratory therapist must be acquired by academic and clinical preparation in a respiratory care program approved by the Commission on Accreditation of Allied Health Education Programs (CAAHEP), or its successor organization, or other accrediting organization recognized by the commissioner or by registry from the National Board for Respiratory Care. Registered respiratory therapists may assume primary clinical

responsibility for all respiratory care modalities. Such modalities must include, but are not limited to, accepting telephone and verbal orders and responsibilities involving the supervision of certified respiratory therapists and respiratory assistants.

(b) Under the supervision of a physician, a registered respiratory therapist may be required to exercise considerable independent clinical judgment in the respiratory care of patients. The registered respiratory therapist must also be capable of serving as a technical resource person to the physician regarding current practices in respiratory care and to the hospital staff regarding effective and safe methods for administering respiratory care.

**63-33-904. Knowledge and skills required of certified respiratory therapists –
Independent clinical judgment prohibited.**

(a) The knowledge and skills for performing the functions of a certified respiratory therapist must be acquired by academic and clinical preparation in a respiratory care program approved by the Commission on Accreditation of Allied Health Education Programs (CAAHEP), or its successor organization, or other accrediting organization recognized by the commissioner or by certification from the National Board for Respiratory Care. Under the supervision of a physician, certified respiratory therapists may assume clinical responsibility for specified respiratory care modalities involving the application of well-defined therapeutic techniques.

(b) The certified respiratory therapist's role must not require the exercising of independent clinical judgment; however, the certified respiratory therapist may be expected to adjust or modify therapeutic techniques within well-defined procedures based on a limited range of patient responses. The effective use of the certified respiratory therapist, especially in the critical care setting, requires the supervision of a physician experienced in respiratory care. The certified respiratory therapist may accept telephone or verbal orders and responsibilities involving the supervision of respiratory care therapists and respiratory assistants.

63-33-905. Knowledge and skills required of respiratory assistants – Independent clinical judgment prohibited.

(a) The knowledge and skills for performing the functions of a respiratory assistant must be acquired through compliance with the requirements of § 63-33-901(a)(4).

(b) The assistant's role must not include the exercising of independent clinical judgment. The assistant may be expected to adjust or modify therapeutic techniques within well-defined procedures based on a limited range of patient responses. The use of the assistant requires the supervision of a registered respiratory therapist, certified respiratory therapist, or physician experienced in respiratory care. An assistant is not permitted to perform the functions for which the assistant is certified on a critical care patient being maintained on mechanical ventilation, unless the assistant possesses a certificate that indicates the assistant has passed the critical care section of the board-approved examination for respiratory assistants.

63-33-906. Exempted persons.

(a)

(1) This part does not prohibit:

(A) A person licensed or certified to practice any of the other health-related professions in this state under any other law from engaging in the practice for which the person is licensed or certified; or

(B) Persons employed as subsidiary workers who do not represent themselves to be registered respiratory therapists, certified respiratory therapists, or respiratory assistants from assisting in the respiratory care of patients under the direction and supervision of a licensed physician.

(2) As used in this subsection (a), "subsidiary worker":

(A) Means an individual who occasionally administers oxygen or other therapeutic gas under the orders of a licensed physician, osteopath, or doctor of dentistry and who is not involved in any other aspect of the direct delivery of respiratory care procedures or the administration of diagnostic tests of the cardiorespiratory function; and

(B) Does not include an individual who delivers and installs respiratory care equipment in hospitals, in the home, and in alternate care sites.

(b) This part does not apply to a registered nurse or to a licensed practical nurse employed by a licensed nursing home when such person is under the supervision of a registered nurse or a licensed physician as long as such licensed practical nurse does not maintain and manage life support systems.

(c) With respect to licensed healthcare professionals who lawfully engage in the practice of respiratory care within the scope of practice of their professions, the commissioner may develop mechanisms and standards for ensuring the competency of such licensed professionals in their practice of respiratory care, and may recommend to the health-related board for each such profession that that board adopt, by rule or otherwise, mechanisms and standards for ensuring competency in the practice of respiratory care; however, the commissioner has no authority to regulate a healthcare professional subject to rule by another health-related board.

(d) This part does not prohibit the practice of respiratory care by students while they are under the supervision of instructors in a respiratory care educational program that has been approved by the Commission on Accreditation of Allied Health Education Programs (CAAHEP), or its successor organization, as long as the students' activities are not performed for compensation and the students wear an appropriate badge indicating their status as students.

(e) This part does not prohibit self-care by the patient or the gratuitous care by a friend or member of the family who does not represent or hold out to be a respiratory care practitioner licensed under this part.

(f) This part does not prohibit an individual who holds a credential from the National Board for Respiratory Care entitled "certified pulmonary function technologist (CPFT)" or "registered pulmonary function technologist (RPFT)" from engaging in the performance of pulmonary function diagnostic testing.

63-33-907. Licensees authorized to perform blood gas analysis.

Blood gas analysis and maintenance and quality assurance of a blood gas laboratory must not be performed by a practitioner licensed under this part unless:

(1) In the case of a registered respiratory therapist or certified respiratory therapist, the registered respiratory therapist or certified respiratory therapist:

(A) Has been trained in blood gas analysis as approved by the commissioner;

(B) Has been granted a license by the commissioner bearing the endorsement "ABG analyst"; and

(C) Holds either the NBRC credentials "CRT," "CRTT," or "RRT" or has passed a commissioner-approved examination testing competency for blood gas analysis; and

(2) In the case of a respiratory assistant:

(A) Has been trained in blood gas analysis as approved by the commissioner;

(B) Has been granted a license by the commissioner bearing the endorsement "ABG analyst"; and

(C) Has passed a commissioner-approved examination testing competency for blood gas analysis. The National Board for Respiratory Care entry level examination, as provided to the commissioner by the

NBRC by contract, is the examination accepted by the commissioner.

This examination does not grant the respiratory assistant the NBRC credential. In addition, the commissioner may write the commissioner's own state council examination or contract with other national testing organizations.

63-33-908. Temporary license.

The commissioner may issue a temporary license to an individual applicant who has completed the required respiratory care educational program but who has not yet successfully completed the NBRC examination. A temporary license shall automatically expire one (1) year from its date of issuance and must not be renewed.

63-33-909. Uniform standards for persons delivering and installing respiratory equipment.

As a part of the board's licensing and regulation of home medical equipment providers under § 68-11-226, the board for licensing health care facilities shall establish uniform standards to ensure the competency of persons who deliver and install respiratory equipment.

SECTION 44. Tennessee Code Annotated, Title 63, Chapter 33, is amended by adding the following as a new part:

63-33-1201. Part definitions.

As used in this part:

(1) "Cosmetic medical service" means a service that uses a biologic or synthetic material, a chemical application, a mechanical device, or a displaced energy form of any kind that alters or damages, or is capable of altering or damaging, living tissue to improve the patient's appearance or achieve an enhanced aesthetic result;

(2) "Media" or "advertising":

(A) Means oral, written, and other types of communication disseminated for the purpose of soliciting medical services; and

(B) Includes newspaper or magazine advertisements, telephone directory displays, printed brochures or leaflets, websites, email correspondence, and television or radio announcements;

(3) "Medical director" means a physician who:

(A) Holds an active medical license under chapter 6 or 9 of this title in this state;

(B) Has an active medical practice in this state; and

(C) Is responsible for the provision of cosmetic medical services;

and

(4) "Medical spa":

(A) Means an entity, however named or organized, that offers or performs cosmetic medical services; and

(B) Does not include an office or practice owned by a physician at which the physician is physically present at least fifty percent (50%) of the medical spa's regular hours of operation.

63-33-1202. Signage required.

(a) An entity doing business as, or advertised as, a medical spa shall display the name of the medical director and indicate one (1) of the following by signage at its practice site and in its media and advertising:

(1) Whether the medical director is certified or eligible for certification by a private or public board, parent association, multidisciplinary board, or association that is a member of the American Board of Medical Specialties (ABMS) or the American Osteopathic Association (AOA);

(2) Whether the medical director is certified by a board or association with equivalent requirements to the ABMS or AOA as approved and recognized

by the board of medical examiners or the board of osteopathic examination, as appropriate; or

(3) Whether the medical director is certified by a board or association requiring an Accreditation Council for Graduate Medical Education (ACGME) or AOA-approved training program that provides complete training in the specialty or subspecialty certified, followed by prerequisite certification by a certifying board of the ABMS or AOA in that training field and successful completion of an additional examination in the specialty or subspecialty certified.

(b) If the medical director is not certified by any of the entities identified in subsection (a), then the lack of certification must be displayed by signage at its practice site and in its media and advertising.

63-33-1203. Regulation by board – Violations – Notice of changes in employment.

(a) Each licensed healthcare practitioner who provides services at a medical spa must continue to be regulated only by the board that has issued a license to that practitioner.

(b) A violation of this part, or a rule promulgated under this part, by a licensed healthcare practitioner while providing services at a medical spa that is required be licensed by this part, is grounds for disciplinary action against that practitioner by the board that licensed the practitioner.

(c) A licensed healthcare practitioner shall notify the board that has licensed the practitioner within ten (10) days of starting or ending work at a medical spa.

63-33-1204. Rules.

In addition to the rulemaking authority conveyed to the commissioner in § 63-33-102, the commissioner shall promulgate rules under this part that address the following topics:

- (1) Personnel requirements for the medical spa;
- (2) Training requirements for healthcare providers;

- (3) Patient records;
- (4) Standards to ensure quality of patient care;
- (5) Infection control;
- (6) Health and safety requirements;
- (7) Data collection and reporting requirements;
- (8) Inspections; and
- (9) Patient billing procedures.

63-33-1205. Documentation required from applicants.

(a) In addition to the requirements of § 63-33-103, an applicant for licensure under this part shall provide the following documentation in the form required by the commissioner:

- (1) That the applicant for the medical spa license is the medical director of the medical spa to be licensed;
- (2) The medical license number of the medical director of the medical spa and whether the medical director is a medical doctor or a doctor of osteopathic medicine;
- (3) The certification information of the medical director as required by § 63-33-1202;
- (4) A list of all medical spas in this state for which the applicant serves as the medical director;
- (5) A list of the names of all owners of the medical spa;
- (6) A list of the names and Tennessee license numbers for employees who will provide cosmetic medical service at the medical spa;
- (7) The location of the medical spa; and
- (8) Any other information the commissioner requires by rule.

(b) A medical spa license is only valid at the location identified in the licensure application. Any changes in location require a new medical spa license, which must be obtained via the initial licensure application process.

(c) A medical spa license is only valid while the original applicant serves as the medical director of the medical spa. If a medical spa changes its medical director, then a new medical spa license must be obtained via the initial licensure application process by the new medical director.

(d) An existing licensed medical spa that changes medical directors may continue to operate on the existing license for sixty (60) days after changing medical directors so that there can be an orderly transition to the newly licensed medical spa.

63-33-1206. Information required from medical directors.

In addition to the updated information required by § 63-33-105(e), the medical director of every licensed medical spa shall also provide the following information to the division within thirty (30) days of their occurrence:

(1) The departure of a person identified in the application for licensure as someone providing cosmetic medical services at the medical spa;

(2) The name and Tennessee license number of a person not identified in the original application form who will provide cosmetic services at the medical spa;

(3) Any change in the ownership of the medical spa; and

(4) Any other information the commissioner requires by rule.

63-33-1207. Investigations by board.

Each board has the authority to investigate a complaint alleging a violation of this part, or a rule adopted under this part, by a medical spa utilizing the services of a healthcare practitioner licensed by that board. Each board also has the authority to investigate a complaint alleging that a medical spa utilizing the services of a healthcare

practitioner licensed by that board is not properly licensed by the division as required by this part.

63-33-1208. Investigations and discipline by commissioner.

The commissioner may investigate and discipline a medical spa license for violations of this part or rules promulgated by the commissioner pursuant to this part, or the reasons listed in § 63-33-106. The commissioner shall not investigate or discipline a healthcare practitioner licensed by a board created under this title or under title 68.

SECTION 45. Tennessee Code Annotated, Section 68-1-101(a)(8), is amended by deleting the subdivision and substituting:

(8) The division of health related boards for all administrative, fiscal, inspectional, and secretarial functions of the following boards, agencies, and commissions:

- (A) Board of alcohol and drug abuse counselors;
- (B) Board for professional counselors, marital and family therapists, and clinical pastoral therapists;
- (C) Board of chiropractic examiners;
- (D) Board of communication disorders and sciences;
- (E) Board of dentistry;
- (F) Board of dietitian/nutritionist examiners;
- (G) Board of examiners for nursing home administrators;
- (H) Board of examiners in psychology;
- (I) Board of medical examiners;
- (J) Board of physician assistants;
- (K) Board of nursing;
- (L) Board of occupational therapy;
- (M) Board of optometry;
- (N) Board of osteopathic examination;

(O) Board of osteopathic examination's council of certified professional midwifery;

(P) Board of pharmacy;

(Q) Board of physical therapy;

(R) Board of podiatric medical examiners;

(S) Board of social worker licensure;

(T) Tennessee medical laboratory board;

(U) Board of veterinary medical examiners; and

(V) Agency of commissioner issued licenses.

SECTION 46. Tennessee Code Annotated, Section 68-1-120, is amended by deleting the section and substituting the following:

By December 31, 2026, and by each December 31 thereafter, the department of health shall publish an annual report stating the total number of attempted suicides and completed suicides that have occurred in this state using the most recent year of final data available. The department of health shall submit the report electronically to all members of the general assembly within seven (7) calendar days of the publication.

SECTION 47. Tennessee Code Annotated, Section 68-1-125(d), is amended by deleting the subsection and substituting:

(d) The department of health, with ongoing consultation of appropriate experts and representatives of relevant providers to provide such consultation, shall determine which of its current programs are evidence-based, research-based, and theory-based. The department of health shall make the measurements of the individual programs, as set forth in subsection (c), available for review on its website.

SECTION 48. Tennessee Code Annotated, Section 68-1-128(h)(4), is amended by deleting the subdivision.

SECTION 49. Tennessee Code Annotated, Section 68-1-128, is amended by adding the following as a new subsection:

(j) The commissioner shall annually report to the chairs of the health and welfare committee of the senate and the committee of the house of representatives having jurisdiction over prescribers of controlled substances a summary of the data concerning prescribers identified pursuant to this section, disciplinary actions taken against those prescribers or taken related to prescription drug abuse or pain management clinics, distribution and abuse of controlled substances, and access to the controlled substance database.

SECTION 50. Tennessee Code Annotated, Section 68-1-147(a)(4), is amended by deleting the subdivision and substituting:

(4) "Palliative care" means specialized care focused on providing relief from physical, psychological, and spiritual suffering, in order to maximize quality of life for both the patient and family, that is provided for people facing a health condition that carries a high risk of mortality, and:

- (A) Negatively impacts a person's daily function;
- (B) Negatively impacts a person's quality of life; or
- (C) Excessively strains the person's caretaker.

SECTION 51. Tennessee Code Annotated, Title 68, Chapter 1, Part 1, is amended by adding the following as a new section:

68-1-150. Rules.

(a) When legislation is enacted by the general assembly that requires a board administratively attached to the department to promulgate rules in a prescribed period of time, the board retains the exclusive authority to promulgate the rules until sixty (60) days before the effective date of the legislation.

(b) If the board has not promulgated emergency rules and scheduled the rulemaking hearing on permanent rules at least sixty (60) days before the effective date of the legislation, the commissioner becomes the exclusive authority to promulgate the rules as described in subsection (a). Upon the receipt of such authority, the

commissioner shall, in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5:

(1) Promulgate emergency rules, in accordance with § 4-5-208, within the time required by the legislation; and

(2) Promulgate permanent rules within one hundred eighty (180) days of the effective date of the emergency rules.

(c) Upon the completion of the requirements of subsection (b), the exclusive authority to promulgate rules reverts to the board. The board may, at that time, amend the permanent rules in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

(d) The rules promulgated by the commissioner in accordance with subsection (b) remain in full force and effect, the same as if promulgated by the board, unless and until the rules are amended by the board.

SECTION 52. Tennessee Code Annotated, Section 68-1-2408, is amended by deleting the section and substituting the following:

(a) Entities receiving grants shall report to the health care and services facility as often as the department determines to be beneficial to program oversight.

(b) The health care and services facility shall report to the department as often as the department determines to be beneficial to program oversight, but at least annually.

(c) The department shall publish annually a summary of program participation and outcome measures.

SECTION 53. Tennessee Code Annotated, Section 68-11-203, is amended by deleting "fifteen (15)" in subdivision (b)(1) and substituting "sixteen (16)"; and by adding the following as a new subdivision (b)(1)(C) and redesignating the existing subdivision (b)(1)(C) and subsequent subdivisions accordingly:

(C) The commissioner of health, or an employee of the department of health, designated by the commissioner;

SECTION 54. Tennessee Code Annotated, Section 68-55-102(b), is amended by deleting the subsection and substituting:

(b) The members are appointed to serve three-year or four-year terms. The successor appointee for the half of the members with a service expiration closest in time after July 1, 2026, shall serve three-year terms, while the other half of the members shall serve four-year terms. Appointment of successors must match the three-year or four-year term of the member they are appointed to succeed.

SECTION 55. The following rules are repealed on July 1, 2026:

- (1) Tenn. Comp. R. & Regs. 0880-11;
- (2) Tenn. Comp. R. & Regs. 0480-01;
- (3) Tenn. Comp. R. & Regs. 0540-01, 0540-02, and 0540-03; and
- (4) Tenn. Comp. R. & Regs. 1200-10-02.

SECTION 56.

(a) The headings in this act are for reference purposes only and do not constitute a part of the law enacted by this act. However, the Tennessee Code Commission is requested to include the headings in any compilation or publication containing this act.

(b) The use of "[former]" and "[repealed]" in this act are for notation purposes only and do not constitute a part of the law enacted by this act. However, the Tennessee Code Commission is requested to include such notations in any compilation or publication containing this act.

(c) The Tennessee Code Commission is requested to reserve parts 7, 10, and 11 in title 63, chapter 33.

SECTION 57. This act takes effect on July 1, 2026, the public welfare requiring it.